



House of Representatives

General Assembly

File No. 529

February Session, 2026

Substitute House Bill No. 5517

House of Representatives, April 9, 2026

The Committee on Public Health reported through REP. MCCARTHY VAHEY of the 133rd Dist., Chairperson of the Committee on the part of the House, that the substitute bill ought to pass.

AN ACT CONCERNING THE DEPARTMENT OF MENTAL HEALTH AND ADDICTION SERVICES' RECOMMENDATIONS REGARDING RECOVERY-FRIENDLY LANGUAGE AND VARIOUS REVISIONS TO MENTAL HEALTH AND ADDICTION STATUTES.

Be it enacted by the Senate and House of Representatives in General Assembly convened:

1 Section 1. Section 4-67s of the general statutes is repealed and the
2 following is substituted in lieu thereof (*Effective October 1, 2026*):

3 As used in sections 4-67s to 4-67x, inclusive, as amended by this act:

4 (1) "Prevention" means policies and programs that promote healthy,
5 safe and productive lives and reduce the likelihood of crime, violence,
6 substance [abuse] use, illness, academic failure and other socially
7 destructive behaviors.

8 (2) "Research-based prevention" means those prevention programs as
9 defined in this section that have been rigorously evaluated and are
10 found to be effective or represent best practices.

11 Sec. 2. Subsection (b) of section 4-67x of the general statutes is
12 repealed and the following is substituted in lieu thereof (*Effective October*
13 *1, 2026*):

14 (b) The ten-year plan shall contain: (1) An identification and analysis
15 of the occurrence of child poverty in the state, (2) an analysis of the long-
16 term effects of child poverty on children, their families and their
17 communities, (3) an analysis of costs of child poverty to municipalities
18 and the state, (4) an inventory of state-wide public and private programs
19 that address child poverty, (5) the percentage of the target population
20 served by such programs and the current state funding levels, if any, for
21 such programs, (6) an identification and analysis of any deficiencies or
22 inefficiencies of such programs, and (7) procedures and priorities for
23 implementing strategies to achieve a fifty per cent reduction in child
24 poverty in the state by June 30, 2014. Such procedures and priorities
25 shall include, but not be limited to, (A) vocational training and
26 placement to promote career progression for parents of children living
27 in poverty, (B) educational opportunities, including higher education
28 opportunities, and advancement for such parents and children,
29 including, but not limited to, preliteracy, literacy and family literacy
30 programs, (C) housing for such parents and children, (D) child care
31 services, as described in section 19a-77, after-school programs and
32 mentoring programs for such children and for single parents, (E) health
33 care access for such parents and children, including access to mental
34 health services and family planning, (F) treatment programs and
35 services, including substance [abuse] use treatment programs and
36 services, for such parents and children, and (G) accessible childhood
37 nutrition programs.

38 Sec. 3. Subsection (a) of section 10-16b of the general statutes is
39 repealed and the following is substituted in lieu thereof (*Effective October*
40 *1, 2026*):

41 (a) In the public schools the program of instruction offered shall
42 include at least the following subject matter, as taught by legally
43 qualified teachers, the arts; career education; consumer education;

44 personal financial management and financial literacy; health and safety,
45 including, but not limited to, human growth and development,
46 nutrition, first aid, including cardiopulmonary resuscitation training in
47 accordance with the provisions of section 10-16qq, disease prevention
48 and cancer awareness, including, but not limited to, age and
49 developmentally appropriate instruction in performing self-
50 examinations for the purposes of screening for breast cancer and
51 testicular cancer, community and consumer health, physical, mental
52 and emotional health, including youth suicide prevention, substance
53 [abuse] use prevention, including instruction relating to opioid use and
54 related disorders, safety, which shall include the safe use of social
55 media, as defined in section 9-601, and may include the dangers of gang
56 membership, and accident prevention; language arts, including reading,
57 writing, grammar, speaking and spelling; mathematics; physical
58 education; science, which may include the climate change curriculum
59 described in subsection (d) of this section; social studies, including, but
60 not limited to, civics and media literacy, citizenship, economics,
61 geography, government, history and Holocaust and genocide education
62 and awareness in accordance with the provisions of section 10-18f;
63 African-American and black studies in accordance with the provisions
64 of section 10-16ss; Puerto Rican and Latino studies in accordance with
65 the provisions of section 10-16ss; Native American studies, in
66 accordance with the provisions of section 10-16vv; Asian American and
67 Pacific Islander studies, in accordance with the provisions of section 10-
68 66ww; computer programming instruction; and in addition, on at least
69 the secondary level, one or more world languages; vocational education;
70 and the black and Latino studies course in accordance with the
71 provisions of sections 10-16tt and 10-16uu. For purposes of this
72 subsection, world languages shall include American Sign Language,
73 provided such subject matter is taught by a qualified instructor under
74 the supervision of a teacher who holds a certificate issued by the State
75 Board of Education. For purposes of this subsection, the "arts" means
76 any form of visual or performing arts, which may include, but not be
77 limited to, dance, music, art and theatre; and "reading" means evidence-
78 based instruction that focuses on competency in oral language,

79 phonemic awareness, phonics, fluency, vocabulary, rapid automatic
80 name or letter name fluency and reading comprehension.

81 Sec. 4. Section 10-19b of the general statutes is repealed and the
82 following is substituted in lieu thereof (*Effective October 1, 2026*):

83 Advisory councils on [drug abuse] substance use education and
84 prevention established by municipalities pursuant to subsection (a) of
85 Section 4126 of the Drug Free Schools and Communities Act of 1986 may
86 serve as a resource for public schools in the field of substance [abuse]
87 use prevention and education and may assist in the development of out-
88 of-school activity for students.

89 Sec. 5. Subsection (a) of section 10-220a of the general statutes is
90 repealed and the following is substituted in lieu thereof (*Effective October*
91 *1, 2026*):

92 (a) Each local or regional board of education shall provide an in-
93 service training program for its teachers, administrators and pupil
94 personnel who hold the initial educator, provisional educator or
95 professional educator certificate. Such program shall provide such
96 teachers, administrators and pupil personnel with information on (1)
97 the nature and the relationship of alcohol and drugs, as defined in
98 section 21a-240, to health and personality development, and procedures
99 for discouraging their abuse, (2) health and mental health risk reduction
100 education that includes, but need not be limited to, the prevention of
101 risk-taking behavior by children and the relationship of such behavior
102 to substance [abuse] use, pregnancy, sexually transmitted diseases,
103 including HIV-infection and AIDS, as defined in section 19a-581, as
104 amended by this act, violence, teen dating violence, domestic violence
105 and child abuse, (3) school violence prevention, conflict resolution and
106 the prevention of and response to youth suicide, provided such school
107 violence prevention training shall be in a manner prescribed in a school
108 security and safety plan, in accordance with the provisions of section 10-
109 222n, (4) cardiopulmonary resuscitation and other emergency life
110 saving procedures, (5) the requirements and obligations of a mandated
111 reporter, (6) the detection and recognition of, and evidence-based

112 structured literacy interventions for, students with dyslexia, as defined
113 in section 10-3d, (7) the laws governing the implementation of planning
114 and placement team meetings and concerning plans pursuant to Section
115 504 of the Rehabilitation Act of 1973, as amended from time to time, (8)
116 an annual update of the new state and federal policies concerning
117 special education, recommendations and best practices, and (9)
118 emergency response to students who experience a seizure in a school,
119 including, but not limited to, the recognition of the signs and symptoms
120 of seizures, the appropriate steps for seizure first aid, information about
121 seizure action plans for students and, for those authorized to administer
122 medication under section 10-212a, the administration of seizure rescue
123 medication or prescribed electrical stimulation using a Vagus Nerve
124 Stimulator magnet. The manner and frequency of the provision of the
125 information described in subdivisions (1) to (9), inclusive, of this
126 subsection shall be determined by the professional development and
127 evaluation committee, established pursuant to subsection (b) of this
128 section, provided such information is provided at least once every five
129 years. Each local or regional board of education may allow any
130 paraeducator or noncertified employee to participate, on a voluntary
131 basis, in any in-service training program provided pursuant to this
132 section.

133 Sec. 6. Subsection (h) of section 14-44k of the general statutes is
134 repealed and the following is substituted in lieu thereof (*Effective October*
135 *1, 2026*):

136 (h) A person is disqualified for life if such person commits two or
137 more of the offenses specified in subsection (b) of this section, or if such
138 person is the subject of two or more findings by the commissioner under
139 subsection (c) of this section, or any combination of those offenses or
140 findings, arising from two or more separate incidents. A person is
141 disqualified for life if the commissioner takes suspension actions against
142 such person for two or more alcohol test refusals or test failures, or any
143 combination of such actions, arising from two or more separate
144 incidents. Any person disqualified for life, except a person disqualified
145 under subsection (g) of this section, who has both voluntarily enrolled

146 in and successfully completed an appropriate rehabilitation program, as
147 determined by the commissioner, may apply for reinstatement of such
148 person's commercial driver's license or commercial learner's permit,
149 provided any such applicant shall not be eligible for reinstatement until
150 such time as such person has served a minimum disqualification period
151 of ten years. An application for reinstatement shall be accompanied by
152 documentation satisfactory to the commissioner that such person has
153 both voluntarily enrolled in and successfully completed a program
154 established and operated by the Department of Mental Health and
155 Addiction Services pursuant to chapter 319j, a program operated
156 through a substance [abuse] use treatment facility licensed in
157 accordance with section 19a-491, as amended by this act, or the
158 equivalent of either program offered in another state. The commissioner
159 shall not reinstate a commercial driver's license or commercial learner's
160 permit that was disqualified for life unless an applicant for
161 reinstatement requests an administrative hearing in accordance with
162 chapter 54, and offers evidence that the reinstatement of such applicant's
163 commercial driver's license or commercial learner's permit does not
164 endanger the public safety or welfare. Such evidence shall include, but
165 not be limited to, proof that such applicant has not been convicted of
166 any offense involving alcohol, a controlled substance or a drug during
167 a period of ten years following the date of such applicant's most recent
168 lifetime disqualification. If a person whose commercial driver's license
169 or commercial learner's permit is reinstated under this subsection is
170 subsequently convicted of another disqualifying offense, such person
171 shall be permanently disqualified for life and shall be ineligible to
172 reapply for a reduction of the lifetime disqualification. The following
173 shall remain on the driving history record of a commercial motor vehicle
174 operator or commercial driver's license or commercial learner's permit
175 holder for a period of fifty-five years, as required by 49 CFR Part 384, as
176 amended from time to time: (1) Any offense specified in subsection (b)
177 or (c) of this section, provided such offense occurred on or after
178 December 29, 2006; (2) each of two or more offenses specified in
179 subsection (b) or (c) of this section that occur within ten years of each
180 other and result in a lifetime disqualification, regardless of when such

181 offenses occur; (3) any conviction under subsection (g) of this section for
182 using a motor vehicle in the commission of a felony involving the
183 manufacture, distribution or dispensing of a controlled substance,
184 committed on or after January 1, 2005.

185 Sec. 7. Subsection (a) of section 17a-4 of the 2026 supplement to the
186 general statutes is repealed and the following is substituted in lieu
187 thereof (*Effective October 1, 2026*):

188 (a) There shall be a State Advisory Council on Children and Families
189 which shall consist of the following members: (1) Nineteen members
190 appointed by the Governor, including two persons who are child care
191 professionals, two persons eighteen to twenty-five years of age,
192 inclusive, served by the Department of Children and Families, one child
193 psychiatrist licensed to practice medicine in this state, one health care
194 professional who has expertise in children's health and is licensed in the
195 state, one attorney who has expertise in legal issues related to children
196 and youth, three members of one or more Youth Advisory Boards, as
197 defined in section 17a-10c, one member of an organization that
198 advocates for the protection and advancement of the legal rights of
199 children, one member of an organization that advocates for policies to
200 promote child welfare and seven persons who shall be representative of
201 young persons, parents and others interested in the delivery of services
202 to children and youths, including child protection, behavioral health
203 and prevention services, at least four of whom shall be parents, foster
204 parents or family members of children who have received, or are
205 receiving, behavioral health services or child welfare services; and (2)
206 six members representing the regional advisory councils established
207 pursuant to section 17a-30, appointed one each by the members of each
208 council. Not more than half the members of the council shall be persons
209 who receive income from a private practice or any public or private
210 agency that delivers mental health, substance [abuse] use, child abuse
211 prevention and treatment or child welfare services. Members of the
212 council shall serve without compensation, except for necessary expenses
213 incurred in the performance of their duties. The Department of Children
214 and Families shall provide the council with funding to facilitate the

215 participation of those members representing families and youth, as well
216 as for other administrative support services. Members shall serve on the
217 council for terms of two years each and no member shall serve for more
218 than three consecutive terms. The commissioner shall be an ex-officio
219 member of the council without vote and shall attend its meetings. Any
220 member who fails to attend three consecutive meetings or fifty per cent
221 of all meetings during any calendar year shall be deemed to have
222 resigned. The council shall elect a chairperson and vice-chairperson to
223 act in the chairperson's absence.

224 Sec. 8. Subsection (a) of section 17a-22g of the general statutes is
225 repealed and the following is substituted in lieu thereof (*Effective October*
226 *1, 2026*):

227 (a) The Judicial Branch and each state agency, community-based
228 program, organization or individual that provides behavioral health or
229 substance [abuse] use prevention and treatment programs that are
230 operated, funded or licensed by the Department of Children and
231 Families pursuant to sections 17a-20, 17a-114, 17a-145, 17a-147, 17a-149,
232 17a-151 and 17a-152 shall provide case specific information to the
233 department for purposes directly connected with the administration of
234 Connecticut Community KidCare in such form and manner as the
235 department requests. The provisions of this section shall be subject to
236 the confidentiality requirements as set forth in applicable federal law.

237 Sec. 9. Subsection (a) of section 17a-22dd of the general statutes is
238 repealed and the following is substituted in lieu thereof (*Effective October*
239 *1, 2026*):

240 (a) Not later than December 1, 2014, the Office of Early Childhood,
241 through the Early Childhood Education Cabinet, shall provide
242 recommendations for implementing the coordination of home visitation
243 programs within the early childhood system that offer a continuum of
244 services to vulnerable families with young children, including
245 prevention, early intervention and intensive intervention, to the joint
246 standing committees of the General Assembly having cognizance of
247 matters relating to appropriations, human services, education and

248 children. Vulnerable families with young children may include, but are
249 not limited to, those facing poverty, trauma, violence, special health care
250 needs, mental, emotional or behavioral health care needs, substance
251 [abuse] use challenges and teen parenthood. The recommendations
252 shall address, at a minimum:

253 (1) A common referral process for families requesting home visitation
254 programs;

255 (2) A core set of competencies and required training for all home
256 visitation program staff;

257 (3) A core set of standards and outcomes for all programs, including
258 requirements for a monitoring framework;

259 (4) Coordinated training for home visitation and early care providers,
260 to the extent that training is currently provided, on cultural competency,
261 mental health awareness and issues such as child trauma, poverty,
262 literacy and language acquisition;

263 (5) Development of common outcomes;

264 (6) Shared reporting of outcomes, including information on any
265 existing gaps in services, disaggregated by agency and program, which
266 shall be reported annually, pursuant to section 11-4a, to the joint
267 standing committees of the General Assembly having cognizance of
268 matters relating to appropriations, human services and children;

269 (7) Home-based treatment options for parents of young children who
270 are suffering from severe depression; and

271 (8) Intensive intervention services for children experiencing mental,
272 emotional or behavioral health issues, including, but not limited to,
273 relationship-focused intervention services for young children.

274 Sec. 10. Subsection (b) of section 17a-62a of the general statutes is
275 repealed and the following is substituted in lieu thereof (*Effective October*
276 *1, 2026*):

277 (b) The Department of Housing, in collaboration with the
278 Department of Children and Families, within available appropriations,
279 shall establish a program that provides one or more of the following
280 services for homeless youth: Public outreach, respite housing, and
281 transitional living services for homeless youth and youth at risk of
282 homelessness. The Department of Housing may enter into a contract
283 with nonprofit organizations or municipalities to implement this
284 section. Such program may have the following components:

285 (1) A public outreach and drop-in component that provides youth
286 drop-in centers with walk-in access to crisis intervention and ongoing
287 supportive services, including one-to-one case management services on
288 a self-referral basis and public outreach that locates, contacts and
289 provides information, referrals and services to homeless youth and
290 youth at risk of homelessness. Such component may include, but need
291 not be limited to, information, referrals and services for (A) family
292 reunification services, conflict resolution or mediation counseling; (B)
293 respite housing, case management aimed at obtaining food, clothing,
294 medical care or mental health counseling, counseling regarding
295 violence, prostitution, substance [abuse] use, sexually transmitted
296 diseases, HIV and pregnancy, and referrals to agencies that provide
297 support services to homeless youth and youth at risk of homelessness;
298 (C) education, employment and independent living skills; (D) aftercare
299 services; and (E) specialized services for highly vulnerable homeless
300 youth, including teen parents, sexually exploited youth and youth with
301 mental illness or developmental disabilities;

302 (2) A respite housing component that provides homeless youth with
303 referrals and walk-in access to respite care on an emergency basis that
304 includes voluntary housing, with private shower facilities, beds and at
305 least one meal each day, and assistance with reunification with family
306 or a legal guardian when required or appropriate. Services provided at
307 respite housing may include, but need not be limited to, (A) family
308 reunification services or referral to safe housing; (B) individual, family
309 and group counseling; (C) assistance in obtaining clothing; (D) access to
310 medical and dental care and mental health counseling; (E) education

311 and employment services; (F) recreational activities; (G) case
312 management, advocacy and referral services; (H) independent living
313 skills training; and (I) aftercare services and transportation; and

314 (3) A transitional living component that (A) assists homeless youth in
315 finding and maintaining safe housing, and (B) includes rental assistance
316 and related supportive services. Such component may include, but need
317 not be limited to, (i) educational assessment and referral to educational
318 programs; (ii) career planning, employment, job skills training and
319 independent living skills training; (iii) job placement; (iv) budgeting and
320 money management; (v) assistance in securing housing appropriate to
321 needs and income; (vi) counseling regarding violence, prostitution,
322 substance [abuse] use, sexually transmitted diseases and pregnancy,
323 referral for medical services or chemical dependency treatment; and
324 (vii) parenting skills, self-sufficiency support services or life skills
325 training and aftercare services.

326 Sec. 11. Subsection (e) of section 17a-101j of the general statutes is
327 repealed and the following is substituted in lieu thereof (*Effective October*
328 *1, 2026*):

329 (e) If, after the investigation is completed, the commissioner
330 determines that a parent or guardian inflicting abuse or neglecting a
331 child is in need of treatment for substance [abuse] use, the commissioner
332 shall refer such person to appropriate treatment services.

333 Sec. 12. Section 17a-101n of the general statutes is repealed and the
334 following is substituted in lieu thereof (*Effective October 1, 2026*):

335 The Department of Children and Families shall collect and analyze
336 data to determine the percentage of the department's cases of child
337 abuse and neglect that involve a parent or guardian with a substance
338 [abuse problem] use disorder and utilize such data to develop strategies
339 to reduce the number of such cases in the future.

340 Sec. 13. Subsection (b) of section 17a-450 of the general statutes is
341 repealed and the following is substituted in lieu thereof (*Effective October*

342 1, 2026):

343 (b) For the purposes of chapter 48, the Department of Mental Health
344 and Addiction Services shall be organized to promote comprehensive,
345 client-based services in the areas of mental health treatment and
346 substance [abuse] use treatment and to ensure the programmatic
347 integrity and clinical identity of services in each area. The department
348 shall perform the functions of: Centralized administration, planning
349 and program development; prevention and treatment programs and
350 facilities, both inpatient and outpatient, for persons with psychiatric
351 disabilities or persons with substance use disorders, or both; community
352 mental health centers and community or regional programs and
353 facilities providing services for persons with psychiatric disabilities or
354 persons with substance use disorders, or both; training and education;
355 and research and evaluation of programs and facilities providing
356 services for persons with psychiatric disabilities or persons with
357 substance use disorders, or both. The department shall include, but not
358 be limited to, the following divisions and facilities or their successor
359 facilities: The office of the Commissioner of Mental Health and
360 Addiction Services; Capitol Region Mental Health Center; Connecticut
361 Valley Hospital, including the Addictions Division and the General
362 Psychiatric Division of Connecticut Valley Hospital; the Whiting
363 Forensic Hospital; the Connecticut Mental Health Center; Ribicoff
364 Research Center; the Southwest Connecticut Mental Health System,
365 including the Franklin S. DuBois Center and the Greater Bridgeport
366 Community Mental Health Center; the Southeastern Mental Health
367 Authority; River Valley Services; the Western Connecticut Mental
368 Health Network; and any other state-operated facility for the treatment
369 of persons with psychiatric disabilities or persons with substance use
370 disorders, or both, but shall not include those portions of such facilities
371 transferred to the Department of Children and Families for the purpose
372 of consolidation of children's services. All department divisions and
373 facilities shall provide their patient records to the electronic health
374 record system established pursuant to subdivision (7) of subsection (c)
375 of this section. Disclosures of patient information from the electronic
376 health record system outside of the department shall be in accordance

377 with applicable federal and state law.

378 Sec. 14. Subsection (d) of section 17a-450 of the general statutes is
379 repealed and the following is substituted in lieu thereof (*Effective October*
380 *1, 2026*):

381 (d) The Department of Mental Health and Addiction Services is
382 designated as the lead state agency for substance [abuse] use prevention
383 and treatment in this state, and as such is designated as the state
384 [methadone] opioid treatment authority. As the designated state
385 [methadone] opioid treatment authority, the department is authorized
386 by the federal Center for Substance Abuse Treatment of the Substance
387 Abuse and Mental Health Services Administration within the United
388 States Department of Health and Human Services to exercise
389 responsibility and authority for the treatment of [opiate addiction]
390 opioid use disorder with an opioid medication, and specifically for: (1)
391 Approval of exceptions to federal opioid treatment protocols in
392 accordance with the Center for Substance Abuse Treatment, (2)
393 monitoring all opioid treatment programs in the state, and (3) approval
394 of Center for Substance Abuse Treatment certification of all opioid
395 treatment programs in the state. The Commissioner of Mental Health
396 and Addiction Services may adopt regulations in accordance with
397 chapter 54 to carry out the provisions of this subsection.

398 Sec. 15. Subsection (a) of section 17a-451 of the general statutes is
399 repealed and the following is substituted in lieu thereof (*Effective October*
400 *1, 2026*):

401 (a) The Commissioner of Mental Health and Addiction Services shall
402 be a qualified person with a master's degree or higher in a health-related
403 field and at least ten years' experience in hospital, health, mental health
404 or substance [abuse] use administration.

405 Sec. 16. Section 17a-453c of the general statutes is repealed and the
406 following is substituted in lieu thereof (*Effective October 1, 2026*):

407 There shall be an interagency collaboration, to be known as "Project

408 Safe", between the Department of Mental Health and Addiction Services
409 and the Department of Children and Families, for the evaluation of and
410 service delivery to families identified by the Department of Children
411 and Families as requiring substance [abuse] use and other behavioral
412 health services. Such collaboration shall include, but not be limited to,
413 evaluations, service needs, service delivery, housing, medical coverage,
414 vocation and employment support and other related recovery support
415 services. The Commissioner of Mental Health and Addiction Services
416 and the Commissioner of Children and Families shall enter into a
417 written memorandum of understanding to carry out the interagency
418 collaboration required under this section. The Department of Social
419 Services and the Labor Department may participate in such
420 collaboration as necessary on a case-by-case basis.

421 Sec. 17. Subsection (a) of section 17a-456 of the general statutes is
422 repealed and the following is substituted in lieu thereof (*Effective October*
423 *1, 2026*):

424 (a) On and before October 1, 2022, there shall be a Board of Mental
425 Health and Addiction Services that shall consist of: (1) Nineteen
426 members appointed by the Governor, subject to the provisions of section
427 4-9a, five of whom shall have had experience in the field of substance
428 use disorders, five of whom shall be from the mental health community,
429 three of whom shall be physicians licensed to practice medicine in this
430 state who have had experience in the field of psychiatry, two of whom
431 shall be psychologists licensed to practice in this state, two of whom
432 shall be persons representing families of individuals with behavioral
433 health disorders, and two of whom shall be persons representing
434 families of individuals recovering from substance use disorders; (2) the
435 chairmen of the regional mental health boards; (3) one designee of each
436 such board; (4) two designees from each of the five subregions
437 represented by the substance abuse subregional planning and action
438 councils; (5) one designee from each mental health region established
439 pursuant to section 17a-478, as amended by this act, each of whom shall
440 represent individuals with psychiatric disabilities, selected by such
441 regional mental health boards in collaboration with advocacy groups;

442 and (6) one designee from each of the five subregions represented by
443 such substance abuse subregional planning and action councils, each of
444 whom shall represent individuals recovering from substance use
445 disorders, selected by such substance [abuse] use subregional planning
446 and action councils in collaboration with advocacy groups. The
447 members of the board shall serve without compensation except for
448 necessary expenses incurred in performing their duties. The members
449 of the board may include representatives of nongovernment
450 organizations or groups, and of state agencies, concerned with
451 planning, operation or utilization of facilities providing mental health
452 and substance use disorder services, including consumers and
453 providers of such services who are familiar with the need for such
454 services, except that no more than half of the members of the board shall
455 be providers of such services. Appointed members shall serve on the
456 board for terms of four years each and members who are designees shall
457 serve on the board at the pleasure of the designating authority. No
458 appointed member of the board shall be employed by the state or be a
459 member of the staff of any institution for which such member's
460 compensation is paid wholly by the state. A majority of the board shall
461 constitute a quorum.

462 Sec. 18. Subsections (f) and (g) of section 17a-457 of the general
463 statutes are repealed and the following is substituted in lieu thereof
464 (*Effective October 1, 2026*):

465 (f) The board shall advise and assist the Commissioner of Mental
466 Health and Addiction Services on program development and
467 community mental health or substance [abuse] use center construction
468 planning.

469 (g) The board is designated and shall serve as the state advisory
470 council to consult with the Department of Mental Health and Addiction
471 Services in administering the state's mental health and substance
472 [abuse] use programs.

473 Sec. 19. Section 17a-464 of the general statutes is repealed and the
474 following is substituted in lieu thereof (*Effective October 1, 2026*):

475 The Ribicoff Research Center is established and shall be operated by
476 the Department of Mental Health and Addiction Services as a facility
477 with state-wide responsibility for research in mental health or substance
478 [abuse] use, or both, to include, but not be limited to, the following
479 areas: Neurochemistry, neurophysiology, clinical behavior and clinical
480 evaluation.

481 Sec. 20. Section 17a-484c of the general statutes is repealed and the
482 following is substituted in lieu thereof (*Effective October 1, 2026*):

483 Any licensed residential treatment facility that provides adult mental
484 health or substance [abuse] use treatment services, or both, and receives
485 state funds for the provision of such services shall prepare a discharge
486 plan, including housing referrals, for each client receiving such services
487 prior to such client's release from such residential treatment facility. The
488 Commissioner of Mental Health and Addiction Services may adopt
489 regulations, in accordance with chapter 54, to carry out the provisions
490 of this section.

491 Sec. 21. Subsection (b) of section 17a-484f of the general statutes is
492 repealed and the following is substituted in lieu thereof (*Effective October*
493 *1, 2026*):

494 (b) The duties of each regional behavioral health action organization,
495 within its mental health region, shall include, but need not be limited to:
496 (1) Assessing the behavioral health needs of children, adolescents and
497 adults across the region and engaging with stakeholders to identify
498 needs, problems, barriers and gaps in the behavioral health service
499 continuum, (2) enhancing the capacity of local communities to
500 understand and address problem gambling, (3) raising awareness and
501 advocating for the general public for mental health promotion and
502 substance [abuse] use prevention, treatment and recovery, (4) receiving
503 and expanding federal, state and local funds and leveraging funds to
504 support behavioral health promotion, prevention, treatment and
505 recovery activities, (5) serving on local, regional and state advisory and
506 planning bodies, (6) within available appropriations, providing training
507 in the administration of an opioid antagonist, as defined in section 17a-

508 714a, and distributing supplies of opioid antagonists to communities,
509 (7) reporting community needs, program review findings and
510 conclusions annually to the relevant local, regional and state
511 stakeholders with recommendations for the establishment, modification
512 or expansion of behavioral health services within the mental health
513 region, and (8) serving as the regional partner responsible for
514 coordinating and aligning federal, state, regional and local behavioral
515 health initiatives.

516 Sec. 22. Subsections (c) to (e), inclusive, of section 17a-485d of the
517 general statutes are repealed and the following is substituted in lieu
518 thereof (*Effective October 1, 2026*):

519 (c) The Commissioner of Social Services shall take such action as may
520 be necessary to amend the Medicaid state plan to provide for coverage
521 of optional adult rehabilitation services supplied by providers of mental
522 health services or substance [abuse] use rehabilitation services for adults
523 with serious and persistent mental illness or who have alcoholism or
524 other substance use disorders, that are certified by the Department of
525 Mental Health and Addiction Services. The Commissioner of Social
526 Services shall adopt regulations, in accordance with the provisions of
527 chapter 54, to implement optional rehabilitation services under the
528 Medicaid program. The commissioner shall implement policies and
529 procedures to administer such services while in the process of adopting
530 such policies or procedures in regulation form, provided notice of
531 intention to adopt the regulations is printed in the Connecticut Law
532 Journal within forty-five days of implementation, and any such policies
533 or procedures shall be valid until the time final regulations are effective.

534 (d) Not later than February 1, 2006, the Commissioner of Mental
535 Health and Addiction Services, in consultation with the Commissioners
536 of Children and Families and Social Services shall report, in accordance
537 with the provisions of section 11-4a, to the joint standing committees of
538 the General Assembly having cognizance of matters relating to public
539 health, human services and appropriations and the budgets of state
540 agencies, on any moneys received by the state as federal Medicaid

541 reimbursement for providing coverage of optional rehabilitation
542 services for children and adults.

543 (e) The Commissioner of Mental Health and Addiction Services shall
544 have the authority to certify providers of mental health or substance
545 [abuse] use rehabilitation services for adults with serious and persistent
546 mental illness or who have alcoholism or other substance use disorders
547 for the purpose of coverage of optional rehabilitation services. The
548 Commissioner of Mental Health and Addiction Services shall adopt
549 regulations, in accordance with the provisions of chapter 54, for
550 purposes of certification of such providers. The commissioner shall
551 implement policies and procedures for purposes of such certification
552 while in the process of adopting such policies or procedures in
553 regulation form, provided notice of intention to adopt the regulations is
554 printed in the Connecticut Law Journal no later than twenty days after
555 implementation and any such policies and procedures shall be valid
556 until the time the regulations are effective.

557 Sec. 23. Subsection (a) of section 17a-485i of the general statutes is
558 repealed and the following is substituted in lieu thereof (*Effective October*
559 *1, 2026*):

560 (a) The Commissioner of Mental Health and Addiction Services shall,
561 within available appropriations, operate a behavioral health recovery
562 program to provide clinical substance [abuse] use treatment, psychiatric
563 treatment and nonclinical recovery support services, which are not
564 covered under the Medicaid program, for individuals with substance
565 use disorders or psychiatric disabilities who are eligible for Medicaid
566 pursuant to Sections 1902(a)(10)(A)(i)(VIII) and 1902(k)(2) of the Social
567 Security Act. Services provided under the program may include, but
568 shall not be limited to, residential substance [abuse] use treatment,
569 recovery support services, peer supports, housing assistance,
570 transportation, food, clothing and personal care items. The Department
571 of Mental Health and Addiction Services shall be responsible for all
572 services and payments related to the provision of the behavioral health
573 recovery support services for eligible recipients.

574 Sec. 24. Subsections (b) and (c) of section 17a-667 of the general
575 statutes are repealed and the following is substituted in lieu thereof
576 (*Effective October 1, 2026*):

577 (b) The council shall consist of the following members: (1) The
578 Secretary of the Office of Policy and Management, or the secretary's
579 designee; (2) the Commissioners of Children and Families, Consumer
580 Protection, Correction, Education, Mental Health and Addiction
581 Services, Public Health, Emergency Services and Public Protection,
582 Aging and Disability Services and Social Services, and the Insurance
583 Commissioner, or their designees; (3) the Chief Court Administrator, or
584 the Chief Court Administrator's designee; (4) the chairperson of the
585 Board of Regents for Higher Education, or the chairperson's designee;
586 (5) the president of The University of Connecticut, or the president's
587 designee; (6) the Chief State's Attorney, or the Chief State's Attorney's
588 designee; (7) the Chief Public Defender, or the Chief Public Defender's
589 designee; (8) the Child Advocate, or the Child Advocate's designee; and
590 (9) the cochairpersons and ranking members of the joint standing
591 committees of the General Assembly having cognizance of matters
592 relating to public health, criminal justice and appropriations, or their
593 designees. The Commissioner of Mental Health and Addiction Services
594 and the Commissioner of Children and Families shall be cochairpersons
595 of the council and may jointly appoint up to seven individuals to the
596 council as follows: (A) Two individuals in recovery from a substance use
597 disorder or representing an advocacy group for individuals with a
598 substance use disorder; (B) a provider of community-based substance
599 [abuse] use services for adults; (C) a provider of community-based
600 substance [abuse] use services for adolescents; (D) an addiction
601 medicine physician; (E) a family member of an individual in recovery
602 from a substance use disorder; and (F) an emergency medicine
603 physician currently practicing in a Connecticut hospital. The
604 cochairpersons of the council may establish subcommittees and
605 working groups and may appoint individuals other than members of
606 the council to serve as members of the subcommittees or working
607 groups. Such individuals may include, but need not be limited to: (i)
608 Licensed alcohol and drug counselors; (ii) pharmacists; (iii) municipal

609 police chiefs; (iv) emergency medical services personnel; and (v)
610 representatives of organizations that provide education, prevention,
611 intervention, referrals, rehabilitation or support services to individuals
612 with substance use disorder or chemical dependency.

613 (c) The council shall review policies and practices of state agencies
614 and the Judicial Department concerning substance [abuse] use
615 treatment programs, substance [abuse] use prevention services, the
616 referral of persons to such programs and services, and criminal justice
617 sanctions and programs and shall develop and coordinate a state-wide,
618 interagency, integrated plan for such programs and services and
619 criminal sanctions.

620 Sec. 25. Subparagraph (B) of subdivision (1) of subsection (c) of
621 section 17a-667a of the 2026 supplement to the general statutes is
622 repealed and the following is substituted in lieu thereof (*Effective October*
623 *1, 2026*):

624 (B) Establishing a publicly accessible electronic information portal, in
625 the form of an Internet web site or application, as a single point of entry
626 for information regarding the availability of (i) beds at a facility in the
627 state for persons in need of medical treatment for (I) [detoxification]
628 withdrawal management for potentially life-threatening symptoms of
629 withdrawal from alcohol or drugs, and (II) rehabilitation or treatment
630 for alcohol dependency, drug dependency or intoxication, and (ii) slots
631 for outpatient treatment using opioid medication that is used to treat
632 opioid use disorder, including methadone and buprenorphine. Such
633 examination shall include the ability of the portal to (I) provide real-time
634 data on the availability of beds and slots, including, but not limited to,
635 the types of beds and slots available, the location of such beds and slots
636 and the wait times, if available, for such beds and slots, and (II) be
637 accessible to the public.

638 Sec. 26. Subsection (e) of section 17a-667a of the 2026 supplement to
639 the general statutes is repealed and the following is substituted in lieu
640 thereof (*Effective October 1, 2026*):

641 (e) The Connecticut Alcohol and Drug Policy Council shall convene
642 a working group to study substance [abuse] use treatment referral
643 programs that have been established by municipal police departments
644 to refer persons with an opioid use disorder or seeking recovery from
645 drug addiction to substance [abuse] use treatment facilities. The
646 working group shall (1) examine such referral programs, (2) identify any
647 barriers faced by such referral programs, and (3) determine the
648 feasibility of implementing such programs on a state-wide basis. Not
649 later than February 1, 2018, the council shall report, in accordance with
650 the provisions of section 11-4a, to the joint standing committees of the
651 General Assembly having cognizance of matters relating to public
652 health and public safety and security regarding the findings of the
653 working group.

654 Sec. 27. Section 17a-670 of the general statutes is repealed and the
655 following is substituted in lieu thereof (*Effective October 1, 2026*):

656 (a) The Department of Mental Health and Addiction Services shall
657 designate substance [abuse] use planning regions within the state. Such
658 regions and the boundaries of such regions may be redesignated by said
659 department as it deems necessary.

660 (b) The department shall designate subregions within each region
661 established pursuant to subsection (a) of this section. The boundaries of
662 such subregions may be redesignated by said department as it deems
663 necessary. Each subregion shall be located entirely within the
664 boundaries of a substance [abuse] use planning region.

665 Sec. 28. Subsection (b) of section 17a-673a of the general statutes is
666 repealed and the following is substituted in lieu thereof (*Effective October*
667 *1, 2026*):

668 (b) A treatment program that provides treatment or [detoxification]
669 withdrawal management services to any person with an opioid use
670 disorder shall (1) educate such person regarding opioid antagonists and
671 the administration thereof at the time such person is admitted to or first
672 receives services from such program, (2) offer education regarding

673 opioid antagonists and the administration thereof to the relatives and
674 significant other of such person if the relatives and significant other have
675 been identified by such person, and (3) if there is a prescribing
676 practitioner affiliated with such program who determines that such
677 person would benefit from access to an opioid antagonist, issue a
678 prescription for or deliver to such person at least one dose of an opioid
679 antagonist at the time such person is admitted to or first receives
680 treatment services from such program.

681 Sec. 29. Subsection (b) of section 17a-683 of the general statutes is
682 repealed and the following is substituted in lieu thereof (*Effective October*
683 *1, 2026*):

684 (b) Any police officer finding a person who appears to be
685 incapacitated by alcohol shall take him into protective custody and have
686 him brought forthwith to a treatment facility which provides medical
687 triage in accordance with regulations adopted pursuant to section 19a-
688 495, as amended by this act, or to a hospital. The police, in detaining the
689 person and in having him brought forthwith to such a treatment facility
690 or a hospital, shall be taking him into protective custody and shall make
691 every reasonable effort to protect his health and safety. In taking the
692 person into protective custody, the detaining officer may take
693 reasonable steps to protect himself. A taking into protective custody
694 under this section is not an arrest. No entry or other record shall be made
695 to indicate that the person has been arrested or charged with a crime.
696 For purposes of this section "medical triage" means a service which
697 provides immediate assessment of symptoms of substance [abuse] use
698 disorder, the immediate care and treatment of these symptoms as
699 necessary, a determination of need for treatment, and assistance in
700 attaining appropriate continued treatment.

701 Sec. 30. Subsection (d) of section 17a-683 of the general statutes is
702 repealed and the following is substituted in lieu thereof (*Effective October*
703 *1, 2026*):

704 (d) If the medical officer determines that the person requires inpatient
705 treatment, the person shall be (1) admitted to, referred to or detained at

706 a treatment facility that provides medical treatment for [detoxification]
707 withdrawal management or a hospital, or (2) committed to a treatment
708 facility operated by the Department of Mental Health and Addiction
709 Services for emergency treatment pursuant to the provisions of section
710 17a-684, as amended by this act. A person treated under subdivision (1)
711 of this subsection shall be admitted as a voluntary patient, or, if
712 necessary, detained for necessary treatment. If such person is referred
713 to another treatment facility or another hospital, the referring facility or
714 hospital shall arrange for his transportation.

715 Sec. 31. Subsection (a) of section 17a-684 of the general statutes is
716 repealed and the following is substituted in lieu thereof (*Effective October*
717 *1, 2026*):

718 (a) A person who is intoxicated at the time of application for
719 commitment pursuant to subsection (b) of this section and who (1) is
720 dangerous to himself or dangerous to others unless committed, (2)
721 needs medical treatment for [detoxification] withdrawal management
722 for potentially life-threatening symptoms of withdrawal from alcohol or
723 drugs or (3) is incapacitated by alcohol, may be committed for
724 emergency treatment to a treatment facility operated by the Department
725 of Mental Health and Addiction Services or a private treatment facility
726 approved by the department to provide emergency treatment. The
727 requirement that a person be intoxicated at the time of application may
728 be waived if a licensed physician determines that the person is in
729 immediate need of medical treatment for [detoxification] withdrawal
730 management for potentially life-threatening withdrawal symptoms. A
731 refusal to undergo treatment shall not constitute evidence of lack of
732 judgment as to the need for treatment.

733 Sec. 32. Subsection (a) of section 17a-710 of the general statutes is
734 repealed and the following is substituted in lieu thereof (*Effective October*
735 *1, 2026*):

736 (a) It shall be the policy of the Department of Mental Health and
737 Addiction Services to develop and implement treatment programs for
738 pregnant women of any age with substance use disorders and their

739 children. The department shall seek private and public funds for such
740 programs. Each program shall, to the extent possible and within
741 available appropriations, offer comprehensive services, including (1)
742 education and prevention programs in high schools and family
743 planning clinics; (2) outreach services to identify pregnant women with
744 substance use disorders early and enroll them in prenatal care and
745 substance [abuse] use treatment programs; (3) case management
746 services; (4) hospital care with substance [abuse] use treatment available
747 in coordination with obstetric services; (5) pediatric care, including
748 therapeutic care for neurologically, behaviorally or developmentally
749 impaired infants; (6) child care for other siblings; (7) classes on parenting
750 skills; (8) home visitation for those who need additional support or who
751 are reluctant to enter a treatment program; (9) access to WIC and other
752 entitlement programs; (10) vocational training for mothers seeking entry
753 to the job market; and (11) a housing component. To the extent possible
754 all services shall be coordinated to be delivered from a centralized
755 location, utilizing medical vans where available and providing
756 transportation assistance when needed.

757 Sec. 33. Subdivision (2) of section 17a-750 of the general statutes is
758 repealed and the following is substituted in lieu thereof (*Effective October*
759 *1, 2026*):

760 (2) "Human services" means services provided to persons or families
761 experiencing difficulty in meeting their basic human needs for (A)
762 physical survival, including their need for food, shelter, clothing and
763 maintenance of minimum income, (B) preparing for and sustaining
764 employment, (C) job readiness, including employment and training
765 programs and child care programs, (D) social support and interaction,
766 especially in time of personal or family crisis, (E) assistance in
767 addressing specific pathologies, such as health, mental health and
768 substance [abuse] use, and (F) access to available appropriate services,
769 such as education, transportation, information and referral services and
770 includes remedial and preventative services targeted to low and
771 moderate income individuals and families, by age group or by specific
772 need;

773 Sec. 34. Subdivision (8) of subsection (a) of section 17a-838 of the 2026
774 supplement to the general statutes is repealed and the following is
775 substituted in lieu thereof (*Effective October 1, 2026*):

776 (8) "Medical setting" means gatherings or gathering places where
777 physical health, mental health, or both are addressed, including, but not
778 limited to, hospitals, clinics, assisted living and rehabilitation facilities,
779 mental health treatment sessions, psychological evaluations, substance
780 [abuse] use treatment sessions, crisis intervention and appointments or
781 other treatment requiring the presence of a doctor, nurse, medical staff
782 or other health care professional; and

783 Sec. 35. Subdivision (4) of subsection (c) of section 17b-28 of the 2026
784 supplement to the general statutes is repealed and the following is
785 substituted in lieu thereof (*Effective October 1, 2026*):

786 (4) Three appointed by the majority leader of the House of
787 Representatives, one of whom shall be an advocate for persons with
788 substance [abuse disabilities] use disorders, one of whom shall be a
789 Medicaid dental provider and one of whom shall be a representative of
790 the for-profit nursing home industry;

791 Sec. 36. Subdivision (2) of subsection (d) of section 17b-59d of the
792 general statutes is repealed and the following is substituted in lieu
793 thereof (*Effective October 1, 2026*):

794 (2) Such request for proposals may require an eligible organization
795 responding to the request to: (A) Have not less than three years of
796 experience operating either a state-wide health information exchange in
797 any state or a regional exchange serving a population of not less than
798 one million that (i) enables the exchange of patient health information
799 among health care providers, patients and other authorized users
800 without regard to location, source of payment or technology, (ii)
801 includes, with proper consent, behavioral health and substance [abuse]
802 use treatment information, (iii) supports transitions of care and care
803 coordination through real-time health care provider alerts and access to
804 clinical information, (iv) allows health information to follow each

805 patient, (v) allows patients to access and manage their health data, and
806 (vi) has demonstrated success in reducing costs associated with
807 preventable readmissions, duplicative testing or medical errors; (B) be
808 committed to, and demonstrate, a high level of transparency in its
809 governance, decision-making and operations; (C) be capable of
810 providing consulting to ensure effective governance; (D) be regulated or
811 administratively overseen by a state government agency; and (E) have
812 sufficient staff and appropriate expertise and experience to carry out the
813 administrative, operational and financial responsibilities of the State-
814 wide Health Information Exchange.

815 Sec. 37. Subsection (c) of section 17b-112 of the 2026 supplement to
816 the general statutes is repealed and the following is substituted in lieu
817 thereof (*Effective October 1, 2026*):

818 (c) A family who is subject to time-limited benefits may petition the
819 Commissioner of Social Services for six-month extensions of such
820 benefits. The commissioner shall grant not more than two extensions to
821 such family who has made a good faith effort to comply with the
822 requirements of the program and despite such effort has a total family
823 income below one hundred per cent of the federal poverty level, or has
824 encountered circumstances preventing employment including, but not
825 limited to: (1) Domestic violence or physical harm to such family's
826 children; or (2) other circumstances beyond such family's control. The
827 commissioner shall disregard ninety dollars of earned income in
828 determining applicable family income. The commissioner may grant a
829 subsequent six-month extension if each adult in the family meets one or
830 more of the following criteria: (A) The adult is precluded from engaging
831 in employment activities due to domestic violence or another reason
832 beyond the adult's control; (B) the adult has two or more substantiated
833 barriers to employment including, but not limited to, the lack of
834 available child care, substance [abuse or addiction] use disorder, severe
835 mental or physical health problems, one or more severe learning
836 disabilities, domestic violence or a child who has a serious physical or
837 behavioral health problem; or (C) the adult is employed and works less
838 than thirty-five hours per week due to (i) a documented medical

839 impairment that limits the adult's hours of employment, provided the
840 adult works the maximum number of hours that the medical condition
841 permits, or (ii) the need to care for a disabled member of the adult's
842 household, provided the adult works the maximum number of hours
843 the adult's caregiving responsibilities permit. Families receiving
844 temporary family assistance shall be notified by the department of the
845 right to petition for such extensions. Notwithstanding the provisions of
846 this section, the commissioner shall not provide benefits under the
847 state's temporary family assistance program to a family that is subject to
848 the thirty-six-month benefit limit and has received benefits beginning
849 on or after October 1, 1996, if such benefits result in that family's
850 receiving more than sixty months of time-limited benefits unless that
851 family experiences domestic violence, as defined in Section 402(a)(7)(B),
852 P.L. 104-193. For the purpose of calculating said sixty-month limit: (I) A
853 month shall count toward the limit if the family receives assistance for
854 any day of the month, provided any months of temporary family
855 assistance received during the public health emergency declared by
856 Governor Ned Lamont related to the COVID-19 pandemic shall not be
857 included, and (II) a month in which a family receives temporary
858 assistance for needy families benefits that are issued from a jurisdiction
859 other than Connecticut shall count toward the limit.

860 Sec. 38. Section 17b-112d of the general statutes is repealed and the
861 following is substituted in lieu thereof (*Effective October 1, 2026*):

862 A person convicted of any offense under federal or state law, on or
863 after August 22, 1996, which (1) is classified as a felony, and (2) has as
864 an element the possession, use or distribution of a controlled substance,
865 as defined in Subsection (6) of 21 USC 802, shall be eligible for benefits
866 pursuant to the temporary assistance for needy families program or the
867 supplemental nutrition assistance program pursuant to the Food and
868 Nutrition Act of 2008, if such person has completed a sentence imposed
869 by a court. A person shall also be eligible for said benefits if such person
870 is satisfactorily serving a sentence of a period of probation or is in the
871 process of completing or has completed a sentence imposed by the court
872 of mandatory participation in a substance [abuse] use treatment

873 program or mandatory participation in a substance [abuse] use testing
874 program.

875 Sec. 39. Subsection (c) of section 17b-191 of the 2026 supplement to
876 the general statutes is repealed and the following is substituted in lieu
877 thereof (*Effective October 1, 2026*):

878 (c) To be eligible for cash assistance under the program, a person shall
879 (1) be (A) eighteen years of age or older; (B) a minor found by a court to
880 be emancipated pursuant to section 46b-150; or (C) under eighteen years
881 of age and the commissioner determines good cause for such person's
882 eligibility, and (2) not have assets exceeding five hundred dollars or, if
883 such person is married, such person and his or her spouse shall not have
884 assets exceeding one thousand dollars. In determining eligibility, the
885 commissioner shall disregard from income (A) all United States
886 Department of Veterans Affairs-administered non-service-connected
887 pension benefits, Aid and Attendance pension benefits and
888 Housebound pension benefits that are granted to a veteran, as defined
889 in section 27-103, or the surviving spouse of such veteran; and (B) any
890 tax refund or advance payment with respect to a refundable credit to
891 the same extent such refund or advance payment would be disregarded
892 under 26 USC 6409 in any federal program or state or local program
893 financed in whole or in part with federal funds. No person who [is] has
894 a substance [abuser] use disorder and refuses or fails to enter available,
895 appropriate treatment shall be eligible for cash assistance under the
896 program until such person enters treatment. No person whose benefits
897 from the temporary family assistance program have terminated as a
898 result of time-limited benefits or for failure to comply with a program
899 requirement shall be eligible for cash assistance under the program.

900 Sec. 40. Subsection (a) of section 17b-194 of the general statutes is
901 repealed and the following is substituted in lieu thereof (*Effective October*
902 *1, 2026*):

903 (a) For the purposes of this section and sections 17b-131, 17b-191 to
904 17b-193, inclusive, as amended by this act, 17b-195, as amended by this
905 act, 17b-197 and 17b-198, (1) an "employable person" means one (A) who

906 is sixteen years of age or older but less than sixty-five years of age; and
907 (B) who has no documented physical or mental impairment prohibiting
908 such person from working or participating in an education, training or
909 other work-readiness program, or who has such an impairment which
910 is expected to last less than two months, as determined by the
911 commissioner; (2) an "unemployable person" means a person who (A) is
912 under sixteen years of age or sixty-five years of age or older or fifty-five
913 years of age or older with a history of chronic unemployment; (B) has a
914 physical or mental impairment prohibiting such person from working
915 or participating in an education, training or other work-readiness
916 program, which is expected to last at least six months, as determined by
917 the commissioner; (C) is pending receipt of supplemental security
918 income, Social Security income or financial assistance through another
919 program administered by the Department of Social Services; (D) is
920 needed to care for a child under two years of age or to care for an
921 incapacitated child or spouse; (E) is a full-time high school student in
922 good standing; or (F) is a VISTA volunteer; and (3) a "transitional
923 person" means one (A) who has a documented physical or mental
924 impairment which prevents employment and is expected to last at least
925 two months, but less than six months, as determined by the
926 commissioner, and who has a recent connection to the labor market,
927 unless circumstances precluded participation in the labor force, as
928 determined by the commissioner; or (B) whose determination of
929 unemployability or disability, as defined by the commissioner, is
930 pending and who provides medical documentation of a severe physical
931 or mental impairment which is expected to last at least six months. A
932 person who [is a substance abuser] has a substance use disorder shall be
933 required to participate in treatment, including counseling, and shall be
934 eligible for assistance while waiting for treatment.

935 Sec. 41. Section 17b-195 of the general statutes is repealed and the
936 following is substituted in lieu thereof (*Effective October 1, 2026*):

937 Notwithstanding any provision of the general statutes, when a
938 person who is ineligible for financial assistance due to his or her
939 employability status is currently in or enters a residential substance

940 [abuse] use treatment facility, the Department of Social Services or the
941 Department of Mental Health and Addiction Services shall pay his or
942 her room and board while at such facility, provided the person is eligible
943 to receive medical assistance. Such assistance shall be paid directly to
944 the treatment facility at a rate established by the Department of Social
945 Services or negotiated by the Department of Mental Health and
946 Addiction Services.

947 Sec. 42. Section 17b-241 of the general statutes is repealed and the
948 following is substituted in lieu thereof (*Effective October 1, 2026*):

949 (a) Any rates established by the Commissioner of Social Services in
950 effect February 1, 1991, for mental health and substance [abuse] use
951 residential facilities shall remain in effect through June 30, 1992, except
952 those which would have been decreased effective July 1, 1991, shall be
953 decreased. Any rate increases made during the fiscal year ending June
954 30, 1993, shall not exceed the most recent annual increase in the
955 consumer price index for urban consumers.

956 (b) Any rates established by the Commissioner of Social Services in
957 effect February 1, 1991, for freestanding [detoxification] withdrawal
958 management centers shall remain in effect through June 30, 1992, except
959 those which would have been decreased effective July 1, 1991, shall be
960 decreased. Any rate increases made during the fiscal years ending June
961 30, 1993, June 30, 1994, and June 30, 1995, shall not exceed the most
962 recent annual increase in the consumer price index for urban consumers.
963 Any freestanding [detoxification] withdrawal management center
964 which has an established rate below the average and, due to a material
965 change in circumstances resulting in financial hardship, is aggrieved by
966 a rate determined pursuant to this subsection may, within ten days of
967 receipt of written notice of such rate from the commissioner, request in
968 writing a hearing on such rate. The commissioner shall, upon the receipt
969 of all documentation necessary to evaluate the request, determine
970 whether there has been such a change in circumstances and shall
971 conduct a hearing if appropriate.

972 Sec. 43. Section 17b-241a of the general statutes is repealed and the

973 following is substituted in lieu thereof (*Effective October 1, 2026*):

974 Notwithstanding any provision of the general statutes, the
975 Commissioner of Social Services may reimburse the Department of
976 Mental Health and Addiction Services for targeted case management
977 services that it provides to its target population, which, for purposes of
978 this section, shall include individuals with severe and persistent
979 psychiatric illness and individuals with [persistent substance
980 dependence] substance use disorder. The Commissioners of Social
981 Services and Mental Health and Addiction Services, in consultation
982 with the Secretary of the Office of Policy and Management, shall ensure
983 that all expenditures for intensive care management eligible for
984 Medicaid reimbursement are submitted to the Centers for Medicare and
985 Medicaid Services.

986 Sec. 44. Subdivision (1) of subsection (a) of section 17b-689c of the
987 general statutes is repealed and the following is substituted in lieu
988 thereof (*Effective October 1, 2026*):

989 (1) The Department of Social Services shall perform an initial
990 assessment in the following areas: Education, employment and training
991 history, basic educational needs and other social service needs,
992 including transportation, child care, child support, domestic violence,
993 substance [abuse] use and mental health.

994 Sec. 45. Subsection (a) of section 17b-694 of the general statutes is
995 repealed and the following is substituted in lieu thereof (*Effective October*
996 *1, 2026*):

997 (a) The Labor Commissioner, in consultation with the Commissioner
998 of Social Services and the Commissioner of Mental Health and
999 Addiction Services, shall administer a grant program, within available
1000 appropriations, to fund employment placement projects for recipients
1001 of state-administered general assistance or recipients of Medicaid who
1002 are eighteen to twenty years of age. A grant may be awarded to (1) a
1003 municipality or group of towns which form a region based on a project
1004 plan providing education, training or other assistance in securing

1005 employment, (2) a private substance [abuse] use or mental health
1006 services provider based on a project plan incorporating job placement
1007 in the treatment process, or (3) a nonprofit organization providing
1008 employment services when no municipality or group of towns elect to
1009 apply for such a grant for a given geographic area. A plan may include
1010 cash incentives as a supplement to wages for recipients who work.

1011 Sec. 46. Section 18-69b of the general statutes is repealed and the
1012 following is substituted in lieu thereof (*Effective October 1, 2026*):

1013 The Department of Correction shall establish rehabilitative
1014 programs, including, but not limited to, substance [abuse] use, academic
1015 and vocational education services and work-release and job training, for
1016 women incarcerated at the York Correctional Institution.

1017 Sec. 47. Subsection (a) of section 18-69c of the general statutes is
1018 repealed and the following is substituted in lieu thereof (*Effective October*
1019 *1, 2026*):

1020 (a) The Commissioner of Correction shall ensure that at least one
1021 departmental or contracted, licensed health care provider who is
1022 employed at the York Correctional Institution (1) has been trained in
1023 prenatal and postpartum medical care, and (2) has knowledge of and
1024 the ability to educate any inmate who is pregnant concerning prenatal
1025 nutrition, high-risk pregnancy and [addiction and substance abuse]
1026 substance use during pregnancy and childbirth.

1027 Sec. 48. Subsection (g) of section 18-69c of the general statutes is
1028 repealed and the following is substituted in lieu thereof (*Effective October*
1029 *1, 2026*):

1030 (g) The York Correctional Institution shall provide a pregnant inmate,
1031 prior to the inmate's release, with counseling and discharge planning to
1032 ensure, to the extent feasible, the continuity of prenatal and pregnancy-
1033 related care, including substance [abuse] use programs and treatment
1034 referrals when deemed appropriate.

1035 Sec. 49. Subsection (a) of section 18-87k of the general statutes is

1036 repealed and the following is substituted in lieu thereof (*Effective October*
1037 *1, 2026*):

1038 (a) The Criminal Justice Policy Advisory Commission shall: (1)
1039 Develop and recommend policies for preventing prison and jail
1040 overcrowding; (2) examine the impact of statutory provisions and
1041 current administrative policies on prison and jail overcrowding and
1042 recommend legislation to the Governor and the General Assembly; (3)
1043 research and gather relevant statistical data and other information
1044 concerning the impact of efforts to prevent prison and jail overcrowding
1045 and make such information available to criminal justice agencies and
1046 members of the General Assembly; (4) advise the undersecretary of the
1047 Criminal Justice Policy and Planning Division on policies and
1048 procedures to promote more effective and cohesive state criminal justice
1049 and juvenile justice systems and to develop and implement the offender
1050 reentry strategy as provided in section 18-81w; (5) monitor
1051 developments throughout the state's criminal justice system; (6) identify
1052 specific needs for reentry services in geographic areas throughout the
1053 state; (7) identify institution-based and community-based programs and
1054 services that effectively address offender needs and reduce recidivism
1055 including, but not limited to, education and training, employment
1056 preparation and job bank, transitional health care, family support,
1057 substance [abuse] use, domestic violence and sexual offender programs
1058 and services; and (8) assist the undersecretary of the Criminal Justice
1059 Policy and Planning Division in developing the recommendations
1060 included in the report and presentation made by the division pursuant
1061 to section 4-68p.

1062 Sec. 50. Section 18-100f of the general statutes is repealed and the
1063 following is substituted in lieu thereof (*Effective October 1, 2026*):

1064 Unless otherwise ordered by the court, whenever an arrested person
1065 charged with the commission of no crime other than a class D or E felony
1066 or a misdemeanor, except a violation of section 53a-60a, 53a-60b, 53a-
1067 60c, 53a-60d, 53a-72a, 53a-73a or 53a-181c, is committed by the court to
1068 the custody of the Commissioner of Correction pursuant to section 54-

1069 64a, the commissioner may release such person to a residence approved
1070 by the Department of Correction subject to such conditions as the
1071 commissioner may impose including, but not limited to, participation in
1072 a substance [abuse] use treatment program and being subject to
1073 electronic monitoring or any other monitoring technology or services.
1074 Any person released pursuant to this section shall remain in the custody
1075 of the commissioner and shall be supervised by employees of the
1076 department during the period of such release. Upon the violation by
1077 such person of any condition of such release, the commissioner may
1078 revoke such release and return such person to confinement in a
1079 correctional facility.

1080 Sec. 51. Section 19a-6d of the general statutes is repealed and the
1081 following is substituted in lieu thereof (*Effective October 1, 2026*):

1082 The Commissioner of Public Health and the Commissioner of Mental
1083 Health and Addiction Services shall, within available appropriations,
1084 develop a tobacco [abuse] use reduction and health plan and shall
1085 submit such plan to the joint standing committees of the General
1086 Assembly having cognizance of matters relating to public health and
1087 appropriations and the budgets of state agencies, not later than April 1,
1088 2001. The plan shall consider and recommend actions to (1) reduce
1089 tobacco and substance [abuse] use, and (2) address the unmet physical
1090 and mental health needs of the state, taking into account the most recent
1091 version of the state health plan prepared by the Department of Public
1092 Health pursuant to section 19a-7.

1093 Sec. 52. Subdivision (2) of subsection (e) of section 19a-6h of the
1094 general statutes is repealed and the following is substituted in lieu
1095 thereof (*Effective October 1, 2026*):

1096 (2) Inventory the state's existing primary care infrastructure,
1097 including, but not limited to, (A) the number of primary care providers
1098 practicing in the state, (B) the total amount of money expended on
1099 public and private primary care services during the last fiscal year, (C)
1100 the number of public and private buildings or offices used primarily for
1101 the rendering of primary care services, including, but not limited to,

1102 hospitals, mental health facilities, dental offices, school-based health
1103 clinics, community-based health centers and academic health centers.
1104 For the purposes of this subdivision, "primary care provider" means any
1105 physician, dentist, nurse, provider of services for persons with
1106 psychiatric disabilities or persons with intellectual disability, or other
1107 person involved in providing primary medical, nursing, counseling, or
1108 other health care, substance [abuse] use or mental health service,
1109 including such services associated with, or under contract to, a health
1110 maintenance organization or medical services plan.

1111 Sec. 53. Subsection (b) of section 19a-7c of the general statutes is
1112 repealed and the following is substituted in lieu thereof (*Effective October*
1113 *1, 2026*):

1114 (b) The contract for pregnant women shall include coverage for: (1)
1115 Physician visits for diagnosis and treatment; (2) prenatal and postnatal
1116 care; and (3) outpatient hospital care; and may include coverage for: (A)
1117 Labor and delivery; (B) laboratory and diagnostic tests; (C) prescription
1118 drugs; (D) physical therapy; (E) mental health and substance [abuse] use
1119 visits; and (F) inpatient care, including mental health and substance
1120 [abuse] use treatment, subject to eighty per cent coinsurance on the first
1121 two thousand five hundred dollars of expenses.

1122 Sec. 54. Section 19a-7e of the general statutes is repealed and the
1123 following is substituted in lieu thereof (*Effective October 1, 2026*):

1124 The Department of Public Health, in consultation with the
1125 Department of Social Services, shall establish a three-year
1126 demonstration program to improve access to health care for uninsured
1127 pregnant women under two hundred fifty per cent of the poverty level.
1128 Services to be covered by the program shall include, but not be limited
1129 to, the professional services of obstetricians, dental care providers,
1130 physician assistants or midwives on the staff of the sponsoring hospital
1131 and community-based providers; services of pediatricians for purposes
1132 of assistance in delivery and postnatal care; dietary counseling; dental
1133 care; substance [abuse] use counseling, and other ancillary services
1134 which may include substance [abuse] use treatment and mental health

1135 services, as required by the patient's condition, history or circumstances;
1136 necessary pharmaceutical and other durable medical equipment during
1137 the prenatal period; and postnatal care, as well as preventative and
1138 primary care for children up to age six in families in the eligible income
1139 level. The program shall encourage the acquisition, sponsorship and
1140 extension of existing outreach activities and the activities of mobile,
1141 satellite and other outreach units. The Commissioner of Public Health
1142 shall issue a request for proposals to Connecticut hospitals. Such request
1143 shall require: (1) An interactive relationship between the hospital,
1144 community health centers, community-based providers and the healthy
1145 start program; (2) provisions for case management; (3) provisions for
1146 financial eligibility screening, referrals and enrollment assistance where
1147 appropriate to the medical assistance program, the healthy start
1148 program or private insurance; and (4) provisions for a formal liaison
1149 function between hospitals, community health centers and other health
1150 care providers. Hospitals participating in the program shall report
1151 monthly to the Departments of Public Health and Social Services or their
1152 designees and annually to the joint standing committees of the General
1153 Assembly having cognizance of matters relating to public health and
1154 human services such information as the departments and the
1155 committees deem necessary.

1156 Sec. 55. Subsection (b) of section 19a-124 of the general statutes is
1157 repealed and the following is substituted in lieu thereof (*Effective October*
1158 *1, 2026*):

1159 (b) The programs shall: (1) Be incorporated into existing human
1160 immunodeficiency virus and hepatitis C outreach and prevention
1161 programs in the selected communities; (2) provide access to free and
1162 confidential exchanges of syringes; (3) provide for safe disposal or
1163 exchange of syringes; (4) provide that first-time applicants to the
1164 program receive an initial packet of syringes, educational material and
1165 a list of drug counseling services; (5) offer education on the human
1166 immunodeficiency virus, hepatitis C, reduction in harm caused by such
1167 viruses, and drug overdose prevention measures and assist program
1168 participants in obtaining drug treatment services; (6) provide referrals

1169 for substance [abuse] use counseling or treatment; and (7) provide
1170 referrals for medical or mental health care.

1171 Sec. 56. Subsection (a) of section 19a-490 of the general statutes is
1172 repealed and the following is substituted in lieu thereof (*Effective October*
1173 *1, 2026*):

1174 (a) "Institution" means a hospital, short-term hospital special hospice,
1175 hospice inpatient facility, residential care home, nursing home facility,
1176 home health care agency, home health aide agency, behavioral health
1177 facility, assisted living services agency, substance [abuse] use treatment
1178 facility, outpatient surgical facility, outpatient clinic, clinical laboratory,
1179 blood collection facility, source plasma donation center, birth center, an
1180 infirmary operated by an educational institution for the care of students
1181 enrolled in, and faculty and employees of, such institution; a facility
1182 engaged in providing services for the prevention, diagnosis, treatment
1183 or care of human health conditions, including facilities operated and
1184 maintained by any state agency; and a residential facility for persons
1185 with intellectual disability licensed pursuant to section 17a-227 and
1186 certified to participate in the Title XIX Medicaid program as an
1187 intermediate care facility for individuals with intellectual disability.
1188 "Institution" does not include any facility for the care and treatment of
1189 persons with mental illness or substance use disorder operated or
1190 maintained by any state agency, except Whiting Forensic Hospital and
1191 the hospital and psychiatric residential treatment facility units of the
1192 Albert J. Solnit Children's Center;

1193 Sec. 57. Section 19a-490h of the general statutes is repealed and the
1194 following is substituted in lieu thereof (*Effective October 1, 2026*):

1195 (a) Each hospital licensed by the Department of Public Health as a
1196 short-term general hospital, outpatient surgical facility or outpatient
1197 clinic shall include in the record of each trauma patient a notation
1198 indicating the extent and outcome of screening for alcohol and
1199 substance [abuse] use disorder. For purposes of this section, "trauma
1200 patient" means a patient of sufficient age to be at risk of alcohol and
1201 substance [abuse] use disorder with a traumatic injury, as defined in the

1202 most recent edition of the International Classification of Disease, who is
1203 admitted to the hospital on an inpatient basis, is transferred to or from
1204 an acute care setting, dies or requires emergent trauma team activation.

1205 (b) Each such hospital shall establish protocols for screening patients
1206 for alcohol and substance [abuse] use disorder.

1207 (c) The Department of Mental Health and Addiction Services, after
1208 consultation with the Department of Public Health, shall assist each
1209 hospital required to conduct alcohol and substance [abuse] use disorder
1210 screening pursuant to subsections (a) and (b) of this section with the
1211 development and implementation of alcohol and substance [abuse] use
1212 disorder screening protocols.

1213 Sec. 58. Subdivision (1) of subsection (a) of section 19a-490q of the
1214 general statutes is repealed and the following is substituted in lieu
1215 thereof (*Effective October 1, 2026*):

1216 (1) "Health care employer" means any institution, as defined in
1217 section 19a-490, as amended by this act, with fifty or more full or part-
1218 time employees. "Health care employer" includes a facility for the care
1219 or treatment of mentally ill persons or persons with substance [abuse
1220 issues] use disorder, a residential facility for persons with intellectual
1221 disability licensed pursuant to section 17a-227, and a community health
1222 center, as defined in section 19a-490a; and

1223 Sec. 59. Subsection (a) of section 19a-491 of the general statutes is
1224 repealed and the following is substituted in lieu thereof (*Effective October*
1225 *1, 2026*):

1226 (a) No person acting individually or jointly with any other person
1227 shall establish, conduct, operate or maintain an institution in this state
1228 without a license as required by this chapter, except for persons issued
1229 a license by the Commissioner of Children and Families pursuant to
1230 section 17a-145 for the operation of (1) a substance [abuse] use treatment
1231 facility, or (2) a facility for the purpose of caring for women during
1232 pregnancies and for women and their infants following such

1233 pregnancies, provided such exception shall not apply to the hospital and
1234 psychiatric residential treatment facility units of the Albert J. Solnit
1235 Children's Center. Application for such license shall (A) be made to the
1236 Department of Public Health upon forms provided by it, (B) be
1237 accompanied by the fee required under subsection (c), (d) or (e) of this
1238 section, (C) contain such information as the department requires, which
1239 may include affirmative evidence of ability to comply with reasonable
1240 standards and regulations prescribed under the provisions of this
1241 chapter, and (D) not be required to be notarized. The commissioner may
1242 require as a condition of licensure that an applicant sign a consent order
1243 providing reasonable assurances of compliance with the Public Health
1244 Code. The commissioner may issue more than one chronic disease
1245 hospital license to a single institution until such time as the state offers
1246 a rehabilitation hospital license.

1247 Sec. 60. Subsection (d) of section 19a-495 of the general statutes is
1248 repealed and the following is substituted in lieu thereof (*Effective October*
1249 *1, 2026*):

1250 (d) The Commissioner of Public Health, in consultation with the
1251 Commissioner of Mental Health and Addiction Services, may
1252 implement policies and procedures, in compliance with federal law,
1253 permitting licensed health care providers with prescriptive authority to
1254 prescribe medications to treat persons [dependent on opiates] with
1255 opioid use disorder in freestanding substance [abuse] use treatment
1256 facilities, licensed under section 19a-490, as amended by this act, while
1257 in the process of adopting such policies and procedures in regulation
1258 form, provided the commissioner prints notice of the intent to adopt
1259 regulations in the Connecticut Law Journal not later than thirty days
1260 after the date of implementation of such policies and procedures.
1261 Policies and procedures implemented pursuant to this subsection shall
1262 be valid until the time final regulations are adopted.

1263 Sec. 61. Section 19a-495c of the general statutes is repealed and the
1264 following is substituted in lieu thereof (*Effective October 1, 2026*):

1265 A substance [abuse] use treatment facility licensed as an institution

1266 pursuant to section 19a-490, as amended by this act, and providing
1267 medication-assisted treatment for opioid [addiction] use disorder shall
1268 be permitted to provide methadone delivery and related substance use
1269 treatment services to persons in a nursing home facility licensed
1270 pursuant to section 19a-493. The Department of Public Health may
1271 allow the delivery of methadone and related substance use treatment
1272 services to a nursing home facility if the Commissioner of Public Health
1273 determines that such delivery would not endanger the health, safety or
1274 welfare of any patient. No such delivery shall be conducted unless a
1275 substance [abuse] use treatment facility proposing the delivery of
1276 methadone and related substance use treatment services has made a
1277 request for such delivery in a form and manner prescribed by the
1278 commissioner and the commissioner has approved such request. Upon
1279 approving a request, the commissioner may impose conditions that
1280 assure the health, safety or welfare of any patient. The commissioner
1281 may revoke the approval of a request upon a finding that the health,
1282 safety or welfare of any patient has been jeopardized.

1283 Sec. 62. Section 19a-509e of the general statutes is repealed and the
1284 following is substituted in lieu thereof (*Effective October 1, 2026*):

1285 Each hospital shall establish and implement, on or before October 1,
1286 1992, a protocol whereby each patient who shows symptoms of
1287 substance [abuse] use disorder, shall be provided with informational
1288 referrals to (1) entitlement programs for which the patient may be
1289 eligible; (2) area substance [abuse] use treatment programs; and (3)
1290 appropriate community-based support services.

1291 Sec. 63. Subdivisions (11) and (12) of section 19a-581 of the general
1292 statutes are repealed and the following is substituted in lieu thereof
1293 (*Effective October 1, 2026*):

1294 (11) "Health facility" means an institution, as defined in section 19a-
1295 490, as amended by this act, blood bank, blood center, sperm bank,
1296 organ or tissue bank, clinical laboratory or facility providing care or
1297 treatment to persons with psychiatric disabilities or persons with
1298 intellectual disability or a facility for the treatment of substance [abuse]

1299 use disorder;

1300 (12) "Health care provider" means any physician, physician assistant,
1301 dentist, nurse, provider of services for persons with psychiatric
1302 disabilities or persons with intellectual disability or other person
1303 involved in providing medical, nursing, counseling, or other health
1304 care, substance [abuse] use or mental health service, including such
1305 services associated with, or under contract to, a health maintenance
1306 organization or medical services plan;

1307 Sec. 64. Subdivision (10) of section 19a-630 of the 2026 supplement to
1308 the general statutes is repealed and the following is substituted in lieu
1309 thereof (*Effective October 1, 2026*):

1310 (10) "Health care facility" means (A) hospitals licensed by the
1311 Department of Public Health under chapter 368v; (B) specialty hospitals;
1312 (C) freestanding emergency departments; (D) outpatient surgical
1313 facilities, as defined in section 19a-493b and licensed under chapter
1314 368v; (E) a hospital or other facility or institution operated by the state
1315 that provides services that are eligible for reimbursement under Title
1316 XVIII or XIX of the federal Social Security Act, 42 USC 301, as amended;
1317 (F) a central service facility; (G) mental health facilities; (H) substance
1318 [abuse] use treatment facilities; and (I) any other facility requiring
1319 certificate of need review pursuant to subsection (a) of section 19a-638,
1320 as amended by this act. "Health care facility" includes any parent
1321 company, subsidiary, affiliate or joint venture, or any combination
1322 thereof, of any such facility.

1323 Sec. 65. Subdivision (5) of subsection (a) of section 19a-638 of the
1324 general statutes is repealed and the following is substituted in lieu
1325 thereof (*Effective October 1, 2026*):

1326 (5) The termination of inpatient or outpatient services offered by a
1327 hospital, including, but not limited to, the termination by a short-term
1328 acute care general hospital or children's hospital of inpatient and
1329 outpatient mental health and substance [abuse] use services;

1330 Sec. 66. Section 19a-902 of the general statutes is repealed and the
1331 following is substituted in lieu thereof (*Effective October 1, 2026*):

1332 On or before January 1, 2011, the Department of Public Health, in
1333 consultation with the Department of Mental Health and Addiction
1334 Services, may (1) amend the department's substance [abuse] use
1335 treatment regulations; (2) implement a dual licensure program for
1336 behavioral health care providers who provide both mental health
1337 services and substance [abuse] use services; or (3) permit the use of
1338 saliva-based drug screening or urinalysis when conducting initial and
1339 subsequent drug screenings of persons who [abuse] use substances
1340 other than alcohol at facilities which are licensed by the Department of
1341 Public Health.

1342 Sec. 67. Section 20-14i of the 2026 supplement to the general statutes
1343 is repealed and the following is substituted in lieu thereof (*Effective*
1344 *October 1, 2026*):

1345 Notwithstanding any provisions of chapter 378, said chapter shall not
1346 prohibit the administration of medication to persons (1) attending day
1347 programs, residing in residential facilities or receiving individual and
1348 family support, under the jurisdiction of the Departments of Children
1349 and Families, Correction, Developmental Services and Mental Health
1350 and Addiction Services, (2) being detained in juvenile residential centers
1351 or residing in residential facilities dually licensed by the Department of
1352 Children and Families and the Department of Public Health, (3) residing
1353 in substance [abuse] use treatment facilities licensed by the Department
1354 of Children and Families pursuant to section 17a-145 when such
1355 medication is administered by trained persons, pursuant to the written
1356 order of a physician licensed under this chapter, a dentist licensed under
1357 chapter 379, an advanced practice registered nurse licensed to prescribe
1358 in accordance with section 20-94a or a physician assistant licensed to
1359 prescribe in accordance with section 20-12d, authorized to prescribe
1360 such medication, or (4) residing in facilities licensed or certified by the
1361 Department of Developmental Services, provided (A) such
1362 administration of medication is performed using an auto injector and

1363 limited to the administration of epinephrine or insulin to treat an acute
1364 allergic reaction or diabetes, and (B) such administration of medication
1365 is performed by a trained person who has received specialized training
1366 in the administration of medication by auto injector, as prescribed by
1367 the Commissioner of Developmental Services. The provisions of this
1368 section shall not apply to institutions, facilities or programs licensed
1369 pursuant to chapter 368v.

1370 Sec. 68. Subdivisions (4) to (6), inclusive, of subsection (a) of section
1371 20-74s of the general statutes are repealed and the following is
1372 substituted in lieu thereof (*Effective October 1, 2026*):

1373 (4) "Practice of alcohol and drug counseling" means (A) the clinical
1374 evaluation by a licensed alcohol and drug counselor of substance use
1375 disorders and co-occurring disorders, including screening, assessment
1376 and diagnosis, treatment planning, counseling, therapy, trauma-
1377 informed care and psychoeducation with individuals, families and
1378 groups in the areas of substance use disorders and co-occurring
1379 disorders, and may include, as appropriate, (i) conducting a substance
1380 use disorder screening or psychosocial history evaluation of an
1381 individual to document the individual's use of drugs prescribed for
1382 pain, other prescribed drugs, illegal drugs and alcohol to determine the
1383 individual's risk for substance [abuse] use disorder, (ii) developing a
1384 preliminary diagnosis for the individual based on such screening or
1385 evaluation, (iii) determining the individual's risk for [abuse] misuse of
1386 drugs prescribed for pain [,] and other prescribed drugs, or use of illegal
1387 drugs and alcohol, (iv) developing a treatment plan and referral options
1388 for the individual to ensure the individual's recovery support needs are
1389 met, and (v) developing and submitting an opioid use consultation
1390 report to an individual's primary care provider to be reviewed by the
1391 primary care provider and included in the individual's medical record,
1392 or (B) the professional application by a certified alcohol and drug
1393 counselor of methods that assist an individual or group to develop an
1394 understanding of alcohol and drug dependency problems, define goals
1395 and plan action reflecting the individual's or group's interest, abilities
1396 and needs as affected by alcohol and drug dependency problems;

1397 (5) "Private practice of alcohol and drug counseling" means the
1398 independent practice of alcohol and drug counseling by a licensed or
1399 certified alcohol and drug counselor who is self-employed on a full-time
1400 or part-time basis and who is responsible for that independent practice;

1401 (6) "Self-help group" means a voluntary group of persons who offer
1402 peer support to each other in recovering from [an addiction] a substance
1403 use disorder;

1404 Sec. 69. Section 20-74ss of the general statutes is repealed and the
1405 following is substituted in lieu thereof (*Effective October 1, 2026*):

1406 The Commissioner of Public Health may take any disciplinary action
1407 set forth in section 19a-17 against a radiologist assistant for any of the
1408 following reasons: (1) Failure to conform to the accepted standards of
1409 the profession; (2) conviction of a felony; (3) fraud or deceit in obtaining
1410 or seeking reinstatement of a license to practice as a radiologist assistant;
1411 (4) fraud or deceit in the practice of the profession; (5) negligent,
1412 incompetent or wrongful conduct in professional activities; (6) physical,
1413 mental or emotional illness or disorder resulting in an inability to
1414 conform to the accepted standards of the profession; (7) alcohol or
1415 substance [abuse] use; (8) wilful falsification of entries in any hospital,
1416 patient or other record pertaining to the profession; or (9) violation of
1417 any provision of sections 20-74nn to 20-74tt, inclusive, and subsection
1418 (c) of section 19a-14. The commissioner may order a license holder to
1419 submit to a reasonable physical or mental examination if the physical or
1420 mental capacity of the license holder to practice safely is the subject of
1421 an investigation. The commissioner may petition the superior court for
1422 the judicial district of Hartford to enforce such order or any action taken
1423 pursuant to said section 19a-17. The commissioner shall give notice and
1424 an opportunity to be heard on any contemplated action under said
1425 section 19a-17.

1426 Sec. 70. Subsection (b) of section 20-94d of the general statutes is
1427 repealed and the following is substituted in lieu thereof (*Effective October*
1428 *1, 2026*):

1429 (b) Except as provided in this section, for registration periods
1430 beginning on and after October 1, 2014, a licensee applying for license
1431 renewal shall earn a minimum of fifty contact hours of continuing
1432 education within the preceding twenty-four-month period. Such
1433 continuing education shall: (1) Be in an area of the advanced practice
1434 registered nurse's practice; (2) reflect the professional needs of the
1435 licensee in order to meet the health care needs of the public; (3) include
1436 at least five contact hours of training or education in
1437 pharmacotherapeutics; (4) include at least one contact hour of training
1438 or education in each of the following topics: (A) Infectious diseases,
1439 including, but not limited to, acquired immune deficiency syndrome
1440 and human immunodeficiency virus, (B) risk management, (C) sexual
1441 assault, (D) domestic violence, (E) cultural competency, and (F)
1442 substance [abuse] use disorders, including, but not limited to,
1443 prescribing controlled substances and pain management; (5) on and
1444 after January 1, 2016, include not less than two contact hours of training
1445 or education during the first renewal period in which continuing
1446 education is required and not less than once every six years thereafter
1447 on the topic of mental health conditions common to veterans and family
1448 members of veterans, including (A) determining whether a patient is a
1449 veteran or family member of a veteran, (B) screening for conditions such
1450 as post-traumatic stress disorder, risk of suicide, depression and grief,
1451 and (C) suicide prevention training; and (6) on and after January 1, 2020,
1452 may include not less than two contact hours of training or education
1453 during the first renewal period in which continuing education is
1454 required and not less than once every six years thereafter in diagnosing
1455 and treating cognitive or mental health conditions, including, but not
1456 limited to, Alzheimer's disease, dementia, delirium, related cognitive
1457 impairments and geriatric depression. For purposes of this section,
1458 qualifying continuing education activities include, but are not limited
1459 to, courses, including on-line courses, offered or approved by the
1460 American Nurses Association, Connecticut Hospital Association,
1461 Connecticut Nurses Association, Connecticut League for Nursing, a
1462 specialty nursing society or an equivalent organization in another
1463 jurisdiction, an educational offering sponsored by a hospital or other

1464 health care institution or a course offered by a regionally accredited
1465 academic institution or a state or local health department. The
1466 commissioner may grant a waiver of not more than ten contact hours of
1467 continuing education for an advanced practice registered nurse who:
1468 (A) Engages in activities related to the advanced practice registered
1469 nurse's service as a member of the Connecticut State Board of Examiners
1470 for Nursing, established pursuant to section 20-88; or (B) assists the
1471 department with its duties to boards and commissions as described in
1472 section 19a-14.

1473 Sec. 71. Section 20-162cc of the general statutes is repealed and the
1474 following is substituted in lieu thereof (*Effective October 1, 2026*):

1475 The Commissioner of Public Health may take any disciplinary action
1476 set forth in section 19a-17 against a perfusionist for any of the following
1477 reasons: (1) Failure to conform to the accepted standards of the
1478 profession; (2) conviction of a felony; (3) fraud or deceit in obtaining or
1479 seeking reinstatement of a license to practice perfusion; (4) fraud or
1480 deceit in the practice of the profession; (5) negligent, incompetent or
1481 wrongful conduct in professional activities; (6) physical, mental or
1482 emotional illness or disorder resulting in an inability to conform to the
1483 accepted standards of the profession; (7) alcohol or substance [abuse]
1484 use; (8) wilful falsification of entries in any hospital, patient or other
1485 record pertaining to the profession; or (9) violation of any provision of
1486 sections 20-162aa to 20-162cc, inclusive, as amended by this act. The
1487 commissioner may order a license holder to submit to a reasonable
1488 physical or mental examination if the physical or mental capacity of the
1489 license holder to practice safely is the subject of an investigation. The
1490 commissioner may petition the superior court for the judicial district of
1491 Hartford to enforce such order or any action taken pursuant to said
1492 section 19a-17. The commissioner shall give notice and an opportunity
1493 to be heard on any contemplated action under said section 19a-17.

1494 Sec. 72. Section 20-185m of the general statutes is repealed and the
1495 following is substituted in lieu thereof (*Effective October 1, 2026*):

1496 The Commissioner of Public Health may take any disciplinary action

1497 set forth in section 19a-17 against a behavior analyst for any of the
1498 following reasons: (1) Failure to conform to the accepted standards of
1499 the profession; (2) conviction of a felony; (3) fraud or deceit in obtaining
1500 or seeking reinstatement of a license to practice behavior analysis; (4)
1501 fraud or deceit in the practice of behavior analysis; (5) negligent,
1502 incompetent or wrongful conduct in professional activities; (6) physical,
1503 mental or emotional illness or disorder resulting in an inability to
1504 conform to the accepted standards of the profession; (7) alcohol or
1505 substance [abuse] use; or (8) wilful falsification of entries in any hospital,
1506 patient or other record pertaining to behavior analysis. The
1507 commissioner may order a license holder to submit to a reasonable
1508 physical or mental examination if his or her physical or mental capacity
1509 to practice safely is the subject of an investigation. The commissioner
1510 may petition the superior court for the judicial district of Hartford to
1511 enforce such order or any action taken pursuant to section 19a-17. The
1512 commissioner shall give notice and an opportunity to be heard on any
1513 contemplated action under section 19a-17.

1514 Sec. 73. Subsections (a) and (b) of section 20-195dd of the general
1515 statutes are repealed and the following is substituted in lieu thereof
1516 (*Effective October 1, 2026*):

1517 (a) Except as otherwise provided in subsections (c) and (d) of this
1518 section, an applicant for a license as a professional counselor shall
1519 submit evidence satisfactory to the commissioner of having: (1) (A)
1520 Earned a graduate degree in clinical mental health counseling as part of
1521 a program of higher learning accredited by the Council for
1522 Accreditation of Counseling and Related Educational Programs, or a
1523 successor organization, or (B) (i) completed at least sixty graduate
1524 semester hours in counseling or a related mental health field at a
1525 regionally accredited institution of higher education that included
1526 coursework in each of the following areas: (I) Human growth and
1527 development; (II) social and cultural foundations; (III) counseling
1528 theories; (IV) counseling techniques; (V) group counseling; (VI) career
1529 counseling; (VII) appraisals or tests and measurements to individuals
1530 and groups; (VIII) research and evaluation; (IX) professional orientation

1531 to mental health counseling; (X) addiction and substance [abuse] use
1532 counseling; (XI) trauma and crisis counseling; and (XII) diagnosis and
1533 treatment of mental and emotional disorders, (ii) earned from a
1534 regionally accredited institution of higher education a graduate degree
1535 in counseling or a related mental health field, (iii) completed a one-
1536 hundred-hour practicum in counseling taught by a faculty member
1537 licensed or certified as a professional counselor or its equivalent in
1538 another state, and (iv) completed a six-hundred-hour clinical mental
1539 health counseling internship taught by a faculty member licensed or
1540 certified as a professional counselor or its equivalent in another state; (2)
1541 acquired three thousand hours of postgraduate experience under
1542 professional supervision, including a minimum of one hundred hours
1543 of direct professional supervision, in the practice of professional
1544 counseling, performed over a period of not less than two years; and (3)
1545 passed an examination prescribed by the commissioner. The provisions
1546 of subparagraphs (B)(i)(X) to (B)(i)(XII), inclusive, (B)(iii) and (B)(iv) of
1547 this subsection shall not apply to any applicant who, on or before July
1548 1, 2017, was a matriculating student in good standing in a graduate
1549 degree program at a regionally accredited institution of higher
1550 education in one of the fields required under subparagraph (B) of this
1551 subsection.

1552 (b) An applicant for a license as a professional counselor associate
1553 shall submit to the Commissioner of Public Health evidence satisfactory
1554 to the commissioner of having (1) earned a graduate degree in clinical
1555 mental health counseling as part of a program of higher learning
1556 accredited by the Council for Accreditation of Counseling and Related
1557 Educational Programs, or a successor organization, or (2) (A) completed
1558 at least sixty graduate semester hours in counseling or a related mental
1559 health field at a regionally accredited institution of higher education
1560 that included coursework in each of the following areas: Human growth
1561 and development; social and cultural foundations; counseling theories;
1562 counseling techniques; group counseling; career counseling; appraisals
1563 or tests and measurements to individuals and groups; research and
1564 evaluation; professional orientation to mental health counseling;
1565 addiction and substance [abuse] use counseling; trauma and crisis

1566 counseling; and diagnosis and treatment of mental and emotional
1567 disorders, (B) completed a one-hundred-hour practicum in counseling
1568 taught by a faculty member licensed or certified as a professional
1569 counselor or its equivalent in another state, (C) completed a six-
1570 hundred-hour clinical mental health counseling internship taught by a
1571 faculty member licensed or certified as a professional counselor or its
1572 equivalent in another state, and (D) earned from a regionally accredited
1573 institution of higher education a graduate degree in counseling or a
1574 related mental health field. The provisions of subparagraphs (A) to (C),
1575 inclusive, of subdivision (2) of this subsection shall not apply to any
1576 applicant who, on or before July 1, 2022, earned a graduate degree at a
1577 regionally accredited institution of higher education in counseling or a
1578 related mental health field and has accumulated at least three thousand
1579 hours of experience under professional supervision, as defined in
1580 section 20-195aa.

1581 Sec. 74. Section 20-195ee of the general statutes is repealed and the
1582 following is substituted in lieu thereof (*Effective October 1, 2026*):

1583 The Commissioner of Public Health may deny an application of an
1584 individual or take any disciplinary action set forth in section 19a-17
1585 against a professional counselor or professional counselor associate for
1586 any of the following reasons: (1) Failure to conform to the accepted
1587 standards of the profession; (2) conviction of a felony, provided any
1588 action taken is based upon (A) the nature of the conviction and its
1589 relationship to the license holder's ability to safely or competently
1590 practice professional counseling, (B) information pertaining to the
1591 degree of rehabilitation of the license holder, and (C) the time elapsed
1592 since the conviction or release; (3) fraud or deceit in obtaining or seeking
1593 reinstatement of a license to practice professional counseling; (4) fraud
1594 or deceit in the practice of professional counseling; (5) negligent,
1595 incompetent or wrongful conduct in professional activities; (6) physical,
1596 mental or emotional illness or disorder resulting in an inability to
1597 conform to the accepted standards of the profession; (7) alcohol or
1598 substance [abuse] use; (8) wilful falsification of entries in any hospital,
1599 patient or other record pertaining to professional counseling; or (9)

1600 violation of any provision of sections 20-195aa to 20-195dd, inclusive, as
1601 amended by this act, or any regulation adopted pursuant to section 20-
1602 195ff. The commissioner may order a license holder to submit to a
1603 reasonable physical or mental examination if his physical or mental
1604 capacity to practice safely is the subject of an investigation. The
1605 commissioner may petition the superior court for the judicial district of
1606 Hartford to enforce such order or any action taken pursuant to said
1607 section 19a-17. The commissioner shall give notice and an opportunity
1608 to be heard on any contemplated action under said section 19a-17.

1609 Sec. 75. Section 20-195tt of the general statutes is repealed and the
1610 following is substituted in lieu thereof (*Effective October 1, 2026*):

1611 The Commissioner of Public Health may take any disciplinary action
1612 set forth in section 19a-17 against a genetic counselor for any of the
1613 following reasons: (1) Failure to conform to the accepted standards of
1614 the profession; (2) conviction of a felony; (3) fraud or deceit in obtaining
1615 or seeking reinstatement of a license to practice genetic counseling; (4)
1616 fraud or deceit in the practice of genetic counseling; (5) negligent,
1617 incompetent or wrongful conduct in professional activities; (6) physical,
1618 mental or emotional illness or disorder resulting in an inability to
1619 conform to the accepted standards of the profession; (7) alcohol or
1620 substance [abuse] use; or (8) wilful falsification of entries in any hospital,
1621 patient or other record pertaining to genetic counseling. The
1622 commissioner may order a license holder to submit to a reasonable
1623 physical or mental examination if his or her physical or mental capacity
1624 to practice safely is the subject of an investigation. The commissioner
1625 may petition the superior court for the judicial district of Hartford to
1626 enforce such order or any action taken pursuant to section 19a-17. The
1627 commissioner shall give notice and an opportunity to be heard on any
1628 contemplated action under section 19a-17.

1629 Sec. 76. Section 20-195kkk of the general statutes is repealed and the
1630 following is substituted in lieu thereof (*Effective October 1, 2026*):

1631 The Commissioner of Public Health may deny an application of an
1632 individual or take any disciplinary action set forth in section 19a-17

1633 against a music therapist for any of the following reasons: (1) Failure to
1634 conform to the accepted standards of the profession; (2) conviction of a
1635 felony, provided any action taken is based upon (A) the nature of the
1636 conviction and its relationship to the license holder's ability to safely or
1637 competently practice as a music therapist, (B) information pertaining to
1638 the degree of rehabilitation of the license holder, and (C) the time
1639 elapsed since the conviction or release; (3) fraud or deceit in obtaining
1640 or seeking reinstatement of a license to practice music therapy; (4) fraud
1641 or deceit in the practice of music therapy; (5) negligent, incompetent or
1642 wrongful conduct in professional activities; (6) physical, mental or
1643 emotional illness or disorder resulting in an inability to conform to the
1644 accepted standards of the profession; (7) alcohol or substance [abuse]
1645 use; or (8) wilful falsification of entries in any hospital, patient or other
1646 record pertaining to music therapy. The commissioner may order a
1647 license holder to submit to a reasonable physical or mental examination
1648 if such license holder's physical or mental capacity to practice safely is
1649 the subject of an investigation. The commissioner may petition the
1650 superior court for the judicial district of Hartford to enforce such order
1651 or any action taken pursuant to section 19a-17. The commissioner shall
1652 give notice and an opportunity to be heard on any contemplated action
1653 under section 19a-17.

1654 Sec. 77. Section 20-195qqq of the general statutes is repealed and the
1655 following is substituted in lieu thereof (*Effective October 1, 2026*):

1656 The Commissioner of Public Health may deny an application of an
1657 individual or take any disciplinary action set forth in section 19a-17
1658 against an art therapist for any of the following reasons: (1) Failure to
1659 conform to the accepted standards of the profession; (2) conviction of a
1660 felony, provided any action taken is based upon (A) the nature of the
1661 conviction and its relationship to the license holder's ability to safely or
1662 competently practice as an art therapist, (B) information pertaining to
1663 the degree of rehabilitation of the license holder, and (C) the time
1664 elapsed since the conviction or release; (3) fraud or deceit in obtaining
1665 or seeking reinstatement of a license to practice art therapy; (4) fraud or
1666 deceit in the practice of art therapy; (5) negligent, incompetent or

1667 wrongful conduct in professional activities; (6) physical, mental or
1668 emotional illness or disorder resulting in an inability to conform to the
1669 accepted standards of the profession; (7) alcohol or substance [abuse]
1670 use; or (8) wilful falsification of entries in any hospital, patient or other
1671 record pertaining to art therapy. The commissioner may order a license
1672 holder to submit to a reasonable physical or mental examination if his
1673 or her physical or mental capacity to practice safely is the subject of an
1674 investigation. The commissioner may petition the superior court for the
1675 judicial district of Hartford to enforce such order or any action taken
1676 pursuant to section 19a-17. The commissioner shall give notice and an
1677 opportunity to be heard on any contemplated action under section 19a-
1678 17.

1679 Sec. 78. Subsection (h) of section 20-206bb of the general statutes is
1680 repealed and the following is substituted in lieu thereof (*Effective October*
1681 *1, 2026*):

1682 (h) Notwithstanding the provisions of subsection (a) of this section,
1683 any person who maintains certification with the National Acupuncture
1684 Detoxification Association may practice the five-point auricular
1685 acupuncture protocol specified as part of such certification program as
1686 an adjunct therapy for the treatment of alcohol and drug abuse and
1687 other behavioral interventions for which the protocol is indicated,
1688 provided the treatment is performed under the supervision of a
1689 physician licensed under chapter 370, a physician assistant licensed
1690 under chapter 370, an advanced practice registered nurse licensed under
1691 chapter 378 or an acupuncturist licensed under chapter 384c and is
1692 performed in (1) a private freestanding facility licensed by the
1693 Department of Public Health that provides care or treatment for
1694 [substance abusive or dependent persons] persons with substance use
1695 disorder, (2) a setting operated by the Department of Mental Health and
1696 Addiction Services, or (3) any other setting where such protocol is an
1697 appropriate adjunct therapy to a substance [abuse] use or behavioral
1698 health treatment program. The Commissioner of Public Health may
1699 adopt regulations, in accordance with the provisions of chapter 54, to
1700 implement the provisions of this section.

1701 Sec. 79. Section 20-206nn of the general statutes is repealed and the
1702 following is substituted in lieu thereof (*Effective October 1, 2026*):

1703 The Commissioner of Public Health may take any disciplinary action
1704 set forth in section 19a-17 against a paramedic, emergency medical
1705 technician, emergency medical responder, advanced emergency
1706 medical technician or emergency medical services instructor for any of
1707 the following reasons: (1) Failure to conform to the accepted standards
1708 of the profession; (2) conviction of a felony, in accordance with the
1709 provisions of section 46a-80; (3) fraud or deceit in obtaining or seeking
1710 reinstatement of a license to practice paramedicine or a certificate to
1711 practice as an emergency medical technician, emergency medical
1712 responder, advanced emergency medical technician or emergency
1713 medical services instructor; (4) fraud or deceit in the practice of
1714 paramedicine, the provision of emergency medical services or the
1715 provision of emergency medical services education; (5) negligent,
1716 incompetent or wrongful conduct in professional activities; (6) physical,
1717 mental or emotional illness or disorder resulting in an inability to
1718 conform to the accepted standards of the profession; (7) alcohol or
1719 substance [abuse] use; or (8) wilful falsification of entries in any hospital,
1720 patient or other health record. The commissioner may take any such
1721 disciplinary action against emergency medical services personnel for
1722 violation of any provision of section 20-206mm or any regulations
1723 adopted pursuant to section 20-206oo. The commissioner may order a
1724 license or certificate holder to submit to a reasonable physical or mental
1725 examination if his or her physical or mental capacity to practice safely is
1726 the subject of an investigation. The commissioner may petition the
1727 superior court for the judicial district of Hartford to enforce such order
1728 or any action taken pursuant to section 19a-17. The commissioner shall
1729 give notice and an opportunity to be heard on any contemplated action
1730 under said section 19a-17.

1731 Sec. 80. Subsection (h) of section 20-660 of the general statutes is
1732 repealed and the following is substituted in lieu thereof (*Effective October*
1733 *1, 2026*):

1734 (h) The provisions of this section do not apply to any person licensed
1735 in this state to provide medical, dental, nursing, counseling or other
1736 health care, substance [abuse] use or mental health services.

1737 Sec. 81. Subsection (a) of section 21a-252 of the general statutes is
1738 repealed and the following is substituted in lieu thereof (*Effective October*
1739 *1, 2026*):

1740 (a) A physician, in good faith and in the course of the physician's
1741 professional practice only, may prescribe, administer and dispense
1742 controlled substances, or may cause the same to be administered by a
1743 physician assistant, nurse or intern under the physician's direction and
1744 supervision, for demonstrable physical or mental disorders but not for
1745 drug dependence except in accordance with state and federal laws and
1746 regulations adopted thereunder. Notwithstanding the provisions of this
1747 subsection the Department of Consumer Protection may approve
1748 protocols allowing the dispensing of take-home doses of methadone, by
1749 a registered nurse or licensed practical nurse, to outpatients in duly
1750 licensed substance [abuse] use treatment facilities. Such dispensing shall
1751 be done pursuant to the order of a licensed prescribing practitioner and
1752 using computerized dispensing equipment into which bulk supplies of
1753 methadone are dispensed by a pharmacist. The quantity of methadone
1754 dispensed by such nurse shall not exceed at any one time that amount
1755 allowed under federal or state statutes or regulations governing the
1756 treatment of drug dependent patients. The Department of Consumer
1757 Protection shall conduct inspections of such treatment facilities to
1758 ensure that the computerized dispensing equipment and related
1759 dispensing procedures documented in the approved protocols are
1760 adhered to.

1761 Sec. 82. Subsection (a) of section 21a-274a of the general statutes is
1762 repealed and the following is substituted in lieu thereof (*Effective October*
1763 *1, 2026*):

1764 (a) There is established a drug enforcement grant program which
1765 shall be administered by the Office of Policy and Management. Grants
1766 may be made to municipalities, the Department of Emergency Services

1767 and Public Protection and the Division of Criminal Justice for the
1768 purpose of enforcing federal and state laws concerning controlled
1769 substances, undertaking crime prevention activities related to the
1770 enforcement of such laws, substance [abuse] use prevention education
1771 or training related to such enforcement or education activities. The
1772 Secretary of the Office of Policy and Management shall adopt
1773 regulations in accordance with chapter 54 for the administration of this
1774 subsection, including the establishment of priorities, program
1775 categories, eligibility requirements, funding limitations and the
1776 application process. Such regulations shall provide that the costs of a
1777 community-based police program, as defined in the regulations, may be
1778 paid from a grant made under this section.

1779 Sec. 83. Subdivision (2) of subsection (a) of section 21a-279 of the
1780 general statutes is repealed and the following is substituted in lieu
1781 thereof (*Effective October 1, 2026*):

1782 (2) For a second offense of subdivision (1) of this subsection, the court
1783 shall evaluate such person and, if the court determines such person is a
1784 drug-dependent person, the court may suspend prosecution of such
1785 person and order such person to undergo a substance [abuse] use
1786 treatment program.

1787 Sec. 84. Subdivision (2) of subsection (e) of section 21a-279a of the
1788 general statutes is repealed and the following is substituted in lieu
1789 thereof (*Effective October 1, 2026*):

1790 (2) For an offense under subdivision (1) of this subsection, the court
1791 shall evaluate such person and, if the court determines such person is a
1792 drug-dependent person, the court may suspend prosecution of such
1793 person and order such person to undergo a substance [abuse] use
1794 treatment program.

1795 Sec. 85. Subsection (b) of section 21a-322 of the general statutes is
1796 repealed and the following is substituted in lieu thereof (*Effective October*
1797 *1, 2026*):

1798 (b) If a practitioner dispenses, administers or prescribes any
1799 controlled substance to a patient, the practitioner shall make available
1800 to the Department of Consumer Protection, for inspection by the
1801 department, records of medical evaluations associated with dispensing,
1802 administering or prescribing such controlled substance. Such records
1803 shall be confidential and not be subject to disclosure under the Freedom
1804 of Information Act, as defined in section 1-200. The department may
1805 inspect such records solely for the purpose of investigating any violation
1806 or suspected violation, or enforcing any provision, of this chapter or any
1807 regulation promulgated under this chapter. Nothing in this subsection
1808 shall be construed to require disclosure of any substance [abuse] use
1809 treatment record that is protected from disclosure under 42 USC 290dd-
1810 2, as amended from time to time, or other applicable federal law.

1811 Sec. 86. Subsection (b) of section 21a-420f of the 2026 supplement to
1812 the general statutes is repealed and the following is substituted in lieu
1813 thereof (*Effective October 1, 2026*):

1814 (b) On and after July 1, 2022, there is established a fund to be known
1815 as the "Cannabis Prevention and Recovery Services Fund". The fund
1816 shall contain any moneys required by law to be deposited in the fund
1817 and shall be held by the Treasurer separate and apart from all other
1818 moneys, funds and accounts. Amounts in the fund may be expended
1819 only pursuant to appropriation by the General Assembly. Any balance
1820 remaining in the fund at the end of any fiscal year shall be carried
1821 forward in the fund for the fiscal year next succeeding. Moneys in the
1822 fund shall be appropriated for the purposes of (1) substance [abuse] use
1823 prevention, treatment and recovery services, which may include, but
1824 need not be limited to, the (A) provision of youth cannabis use
1825 prevention services by the local advisory councils on drug use and
1826 prevention established by municipalities pursuant to subsection (a) of
1827 Section 4126 of the Drug Free Schools and Communities Act of 1986, as
1828 amended from time to time, regional behavioral health action
1829 organizations described in section 17a-484f, as amended by this act, or
1830 youth service bureaus established pursuant to section 10-19m, and (B)
1831 development of a public awareness campaign to raise awareness of the

1832 mental and physical health risks of youth cannabis use and cannabis use
1833 by pregnant persons, and (2) collection and analysis of data regarding
1834 substance use. The Social Equity Council may make recommendations
1835 to any relevant state agency regarding expenditures to be made for the
1836 purposes set forth in this subsection.

1837 Sec. 87. Subsection (b) of section 31-40v of the general statutes is
1838 repealed and the following is substituted in lieu thereof (*Effective October*
1839 *1, 2026*):

1840 (b) The chairman of the Workers' Compensation Commission, in
1841 consultation with the Labor Commissioner and in accordance with the
1842 provisions of chapter 54, shall adopt regulations to carry out the
1843 provisions of this section. The regulations shall (1) prescribe the
1844 membership of safety and health committees to ensure representation
1845 of employees and employers; (2) specify the frequency of committee
1846 meetings; (3) require employers to make, file and maintain adequate
1847 written records of each committee meeting subject to inspection by the
1848 chairman or his authorized designee; (4) require employers to
1849 compensate employee representatives at their regular hourly wage
1850 while the employee representatives are engaged in safety and health
1851 committee training or are attending committee meetings; (5) prescribe
1852 the duties and functions of safety and health committees, which shall
1853 include (A) establishing procedures for workplace safety inspections by
1854 the committee, (B) establishing procedures for investigating all safety
1855 incidents, accidents, illnesses and deaths, (C) evaluating accident and
1856 illness prevention programs, (D) establishing training programs for the
1857 identification and reduction of hazards in the workplace which damage
1858 the reproductive systems of employees, and (E) establishing training
1859 programs to assist committee members in understanding and
1860 identifying the effects of employee substance [abuse] use on workplace
1861 accidents and safety; and (6) prescribe guidelines for the training of
1862 safety and health committee members.

1863 Sec. 88. Subdivision (6) of subsection (a) of section 38a-479qq of the
1864 general statutes is repealed and the following is substituted in lieu

1865 thereof (*Effective October 1, 2026*):

1866 (6) "Health care services" means any care, service or treatment of an
1867 illness or dysfunction of, or injury to, the human body. "Health care
1868 services" includes physician care, inpatient care, hospital surgical
1869 services, emergency medical services, ambulance services, dental care
1870 services, vision care services, mental health care services, substance
1871 [abuse] use disorder services, chiropractic services, podiatric services,
1872 laboratory test services and the provision of medical equipment or
1873 supplies. "Health care services" does not include pharmaceutical
1874 supplies or prescriptions;

1875 Sec. 89. Subsections (a) and (b) of section 38a-488a of the general
1876 statutes are repealed and the following is substituted in lieu thereof
1877 (*Effective October 1, 2026*):

1878 (a) For the purposes of this section:

1879 (1) (A) "Mental or nervous conditions" means mental disorders, as
1880 defined in the most recent edition of the American Psychiatric
1881 Association's "Diagnostic and Statistical Manual of Mental Disorders".

1882 (B) "Mental or nervous conditions" does not include (i) intellectual
1883 disability, (ii) specific learning disorders, (iii) motor disorders, (iv)
1884 communication disorders, (v) caffeine-related disorders, (vi) relational
1885 problems, and (vii) other conditions that may be a focus of clinical
1886 attention, that are not otherwise defined as mental disorders in the most
1887 recent edition of the American Psychiatric Association's "Diagnostic and
1888 Statistical Manual of Mental Disorders".

1889 (2) "Benefits payable" means the usual, customary and reasonable
1890 charges for treatment deemed necessary under generally accepted
1891 medical standards, except that in the case of a managed care plan, as
1892 defined in section 38a-478, "benefits payable" means the payments
1893 agreed upon in the contract between a managed care organization, as
1894 defined in section 38a-478, and a provider, as defined in section 38a-478.

1895 (3) "Acute treatment services" means twenty-four-hour medically

1896 supervised treatment for a substance use disorder, that is provided in a
1897 medically managed or medically monitored inpatient facility.

1898 (4) "Clinical stabilization services" means twenty-four-hour clinically
1899 managed [postdetoxification] post-withdrawal management treatment,
1900 including, but not limited to, relapse prevention, family outreach,
1901 aftercare planning and addiction education and counseling.

1902 (b) Each individual health insurance policy providing coverage of the
1903 type specified in subdivisions (1), (2), (4), (11) and (12) of section 38a-469
1904 delivered, issued for delivery, renewed, amended or continued in this
1905 state shall provide benefits for the diagnosis and treatment of mental or
1906 nervous conditions. Benefits payable include, but need not be limited to:

1907 (1) General inpatient hospitalization, including in state-operated
1908 facilities;

1909 (2) Medically necessary acute treatment services and medically
1910 necessary clinical stabilization services;

1911 (3) General hospital outpatient services, including at state-operated
1912 facilities;

1913 (4) Psychiatric inpatient hospitalization, including in state-operated
1914 facilities;

1915 (5) Psychiatric outpatient hospital services, including at state-
1916 operated facilities;

1917 (6) Intensive outpatient services, including at state-operated facilities;

1918 (7) Partial hospitalization, including at state-operated facilities;

1919 (8) Intensive, home-based or evidence-based services designed to
1920 address specific mental or nervous conditions in a child or adolescent;

1921 (9) Evidence-based family-focused therapy that specializes in the
1922 treatment of juvenile substance use disorders;

- 1923 (10) Short-term family therapy intervention;
- 1924 (11) Nonhospital inpatient [detoxification] withdrawal management;
- 1925 (12) Medically monitored [detoxification] withdrawal management;
- 1926 (13) Ambulatory [detoxification] withdrawal management;
- 1927 (14) Inpatient services at psychiatric residential treatment facilities;
- 1928 (15) Rehabilitation services provided in residential treatment
1929 facilities, general hospitals, psychiatric hospitals or psychiatric facilities;
- 1930 (16) Observation beds in acute hospital settings;
- 1931 (17) Psychological and neuropsychological testing conducted by an
1932 appropriately licensed health care provider;
- 1933 (18) Trauma screening conducted by a licensed behavioral health
1934 professional;
- 1935 (19) Depression screening, including maternal depression screening,
1936 conducted by a licensed behavioral health professional; and
- 1937 (20) Substance use screening conducted by a licensed behavioral
1938 health professional.

1939 Sec. 90. Section 38a-488d of the general statutes is repealed and the
1940 following is substituted in lieu thereof (*Effective October 1, 2026*):

1941 No individual health insurance policy providing coverage of the type
1942 specified in subdivisions (1), (2), (4), (11) and (12) of section 38a-469 that
1943 is delivered, issued for delivery, renewed, amended or continued in this
1944 state on or after January 1, 2020, shall deny coverage for covered
1945 substance [abuse] use disorder services solely because such substance
1946 [abuse] use disorder services were provided pursuant to an order issued
1947 by a court of competent jurisdiction.

1948 Sec. 91. Section 38a-492p of the general statutes is repealed and the
1949 following is substituted in lieu thereof (*Effective October 1, 2026*):

1950 Each insurance company, hospital service corporation, medical
1951 service corporation, health care center, fraternal benefit society or other
1952 entity that delivers, issues for delivery, renews, amends or continues in
1953 this state an individual health insurance policy providing coverage of
1954 the type specified in subdivision (1), (2), (4), (11) or (12) of section 38a-
1955 469 that provides coverage to an insured or enrollee who has been
1956 diagnosed with a substance use disorder, as described in section 17a-
1957 458, shall cover medically necessary, medically monitored inpatient
1958 [detoxification] withdrawal management services and medically
1959 necessary, medically managed intensive inpatient [detoxification]
1960 withdrawal management services provided to the insured or enrollee.
1961 For purposes of this section, ["medically monitored inpatient
1962 detoxification" and "medically managed intensive inpatient
1963 detoxification"] "medically monitored inpatient withdrawal
1964 management" and "medically managed intensive inpatient withdrawal
1965 management" have the same meanings as described in the most recent
1966 edition of the American Society of Addiction Medicine Treatment
1967 Criteria for Addictive, Substance-Related and Co-Occurring Conditions.

1968 Sec. 92. Subsections (a) and (b) of section 38a-514 of the general
1969 statutes are repealed and the following is substituted in lieu thereof
1970 (*Effective October 1, 2026*):

1971 (a) For the purposes of this section:

1972 (1) (A) "Mental or nervous conditions" means mental disorders, as
1973 defined in the most recent edition of the American Psychiatric
1974 Association's "Diagnostic and Statistical Manual of Mental Disorders".

1975 (B) "Mental or nervous conditions" does not include (i) intellectual
1976 disability, (ii) specific learning disorders, (iii) motor disorders, (iv)
1977 communication disorders, (v) caffeine-related disorders, (vi) relational
1978 problems, and (vii) other conditions that may be a focus of clinical
1979 attention, that are not otherwise defined as mental disorders in the most
1980 recent edition of the American Psychiatric Association's "Diagnostic and
1981 Statistical Manual of Mental Disorders".

1982 (2) "Benefits payable" means the usual, customary and reasonable
1983 charges for treatment deemed necessary under generally accepted
1984 medical standards, except that in the case of a managed care plan, as
1985 defined in section 38a-478, "benefits payable" means the payments
1986 agreed upon in the contract between a managed care organization, as
1987 defined in section 38a-478, and a provider, as defined in section 38a-478.

1988 (3) "Acute treatment services" means twenty-four-hour medically
1989 supervised treatment for a substance use disorder, that is provided in a
1990 medically managed or medically monitored inpatient facility.

1991 (4) "Clinical stabilization services" means twenty-four-hour clinically
1992 managed [postdetoxification] post-withdrawal management treatment,
1993 including, but not limited to, relapse prevention, family outreach,
1994 aftercare planning and addiction education and counseling.

1995 (b) Except as provided in subsection (j) of this section, each group
1996 health insurance policy providing coverage of the type specified in
1997 subdivisions (1), (2), (4), (11) and (12) of section 38a-469 delivered,
1998 issued for delivery, renewed, amended or continued in this state shall
1999 provide benefits for the diagnosis and treatment of mental or nervous
2000 conditions. Benefits payable include, but need not be limited to:

2001 (1) General inpatient hospitalization, including in state-operated
2002 facilities;

2003 (2) Medically necessary acute treatment services and medically
2004 necessary clinical stabilization services;

2005 (3) General hospital outpatient services, including at state-operated
2006 facilities;

2007 (4) Psychiatric inpatient hospitalization, including in state-operated
2008 facilities;

2009 (5) Psychiatric outpatient hospital services, including at state-
2010 operated facilities;

- 2011 (6) Intensive outpatient services, including at state-operated facilities;
- 2012 (7) Partial hospitalization, including at state-operated facilities;
- 2013 (8) Intensive, home-based or evidence-based services designed to
2014 address specific mental or nervous conditions in a child or adolescent;
- 2015 (9) Evidence-based family-focused therapy that specializes in the
2016 treatment of juvenile substance use disorders;
- 2017 (10) Short-term family therapy intervention;
- 2018 (11) Nonhospital inpatient [detoxification] withdrawal management;
- 2019 (12) Medically monitored [detoxification] withdrawal management;
- 2020 (13) Ambulatory [detoxification] withdrawal management;
- 2021 (14) Inpatient services at psychiatric residential treatment facilities;
- 2022 (15) Rehabilitation services provided in residential treatment
2023 facilities, general hospitals, psychiatric hospitals or psychiatric facilities;
- 2024 (16) Observation beds in acute hospital settings;
- 2025 (17) Psychological and neuropsychological testing conducted by an
2026 appropriately licensed health care provider;
- 2027 (18) Trauma screening conducted by a licensed behavioral health
2028 professional;
- 2029 (19) Depression screening, including maternal depression screening,
2030 conducted by a licensed behavioral health professional; and
- 2031 (20) Substance use screening conducted by a licensed behavioral
2032 health professional.

2033 Sec. 93. Section 38a-514d of the general statutes is repealed and the
2034 following is substituted in lieu thereof (*Effective October 1, 2026*):

2035 No group health insurance policy providing coverage of the type

2036 specified in subdivisions (1), (2), (4), (11) and (12) of section 38a-469 that
2037 is delivered, issued for delivery, renewed, amended or continued in this
2038 state on or after January 1, 2020, shall deny coverage for covered
2039 substance [abuse] use disorder services solely because such substance
2040 [abuse] use disorder services were provided pursuant to an order issued
2041 by a court of competent jurisdiction.

2042 Sec. 94. Section 38a-518p of the general statutes is repealed and the
2043 following is substituted in lieu thereof (*Effective October 1, 2026*):

2044 Each insurance company, hospital service corporation, medical
2045 service corporation, health care center, fraternal benefit society or other
2046 entity that delivers, issues for delivery, renews, amends or continues in
2047 this state a group health insurance policy providing coverage of the type
2048 specified in subdivision (1), (2), (4), (11) or (12) of section 38a-469 that
2049 provides coverage to an insured or enrollee who has been diagnosed
2050 with a substance use disorder, as described in section 17a-458, shall
2051 cover medically necessary, medically monitored inpatient
2052 [detoxification] withdrawal management services and medically
2053 necessary, medically managed intensive inpatient [detoxification]
2054 withdrawal management services provided to the insured or enrollee.
2055 For purposes of this section, ["medically monitored inpatient
2056 detoxification" and "medically managed intensive inpatient
2057 detoxification"] "medically monitored inpatient withdrawal
2058 management" and "medically managed intensive inpatient withdrawal
2059 management" have the same meanings as described in the most recent
2060 edition of the American Society of Addiction Medicine Treatment
2061 Criteria for Addictive, Substance-Related and Co-Occurring Conditions.

2062 Sec. 95. Subdivision (8) of subsection (a) of section 38a-999 of the
2063 general statutes is repealed and the following is substituted in lieu
2064 thereof (*Effective October 1, 2026*):

2065 (8) Additional protection against unauthorized disclosure of sensitive
2066 health information, which shall include information regarding: Sexually
2067 transmitted diseases; mental health; substance [abuse] use disorder; the
2068 human immunodeficiency virus and acquired immune deficiency

2069 syndrome; and genetic testing, including the fact that an individual has
2070 undergone a genetic test.

2071 Sec. 96. Subsection (a) of section 46a-11b of the general statutes is
2072 repealed and the following is substituted in lieu thereof (*Effective October*
2073 *1, 2026*):

2074 (a) Any physician or surgeon licensed under the provisions of chapter
2075 370, any resident physician or intern in any hospital in this state,
2076 whether or not so licensed, any registered nurse, any person paid for
2077 caring for persons in any facility and any licensed practical nurse,
2078 medical examiner, dental hygienist, dentist, occupational therapist,
2079 optometrist, chiropractor, psychologist, podiatrist, social worker, school
2080 teacher, school principal, school guidance counselor, school counselor,
2081 paraeducator, licensed behavior analyst, mental health professional,
2082 physician assistant, licensed or certified [substance abuse] alcohol and
2083 drug counselor, licensed marital and family therapist, speech and
2084 language pathologist, clergyman, police officer, pharmacist, physical
2085 therapist, licensed professional counselor or sexual assault counselor or
2086 domestic violence counselor, as defined in section 52-146k, who has
2087 reasonable cause to suspect or believe that any person with intellectual
2088 disability or any person who receives services from the Department of
2089 Social Services' Division of Autism Spectrum Disorder Services has been
2090 abused or neglected shall, as soon as practicable but not later than forty-
2091 eight hours after such person has reasonable cause to suspect or believe
2092 that a person with intellectual disability or any person who receives
2093 services from the Department of Social Services' Division of Autism
2094 Spectrum Disorder Services has been abused or neglected, report such
2095 information or cause a report to be made in any reasonable manner to
2096 the commissioner, or the commissioner's designee. An unsuccessful
2097 attempt to make an initial report to the commissioner, or the
2098 commissioner's designee, on a weekend, holiday or after normal
2099 business hours shall not be construed as a violation of this section if
2100 reasonable attempts are made by a person required to report under this
2101 subsection to reach the commissioner, or the commissioner's designee,
2102 as soon as practicable after the initial attempt. The initial report shall be

2103 followed up by a written report not later than five calendar days after
2104 the initial report was made. Any person required to report under this
2105 subsection who fails to make such report shall be fined not more than
2106 five hundred dollars. For purposes of this subsection, "reasonable
2107 manner" and "reasonable attempts" mean efforts that include, but are
2108 not limited to, efforts to reach the commissioner, or the commissioner's
2109 designee, by phone, in person or by electronic mail.

2110 Sec. 97. Subparagraph (E) of subdivision (1) of subsection (f) of
2111 section 46a-170 of the 2026 supplement to the general statutes is
2112 repealed and the following is substituted in lieu thereof (*Effective October*
2113 *1, 2026*):

2114 (E) Develop a plan for mental health, support and substance [abuse]
2115 use programs for individuals identified as victims of trafficking and
2116 those arrested for prostitution in violation of section 53a-82. The plan
2117 shall provide for (i) the diversion of victims of trafficking and
2118 prostitution offenders into community-based treatment and support
2119 services, including, but not limited to, substance [abuse] use recovery,
2120 housing, healthcare, job training, treatment and mental health support,
2121 and (ii) after the successful completion of the program, the dismissal of
2122 any related criminal charges against the accused.

2123 Sec. 98. Subsection (c) of section 46b-38d of the general statutes is
2124 repealed and the following is substituted in lieu thereof (*Effective October*
2125 *1, 2026*):

2126 (c) For the purpose of establishing accurate data on the extent and
2127 severity of family violence in the state and on the degree of compliance
2128 with the requirements of sections 46b-38a to 46b-38f, inclusive, the
2129 Commissioner of Emergency Services and Public Protection shall
2130 prescribe a form for making family violence offense reports. The form
2131 shall include, but is not limited to, the following: (1) Name of the parties;
2132 (2) relationship of the parties; (3) sex of the parties; (4) date of birth of
2133 the parties; (5) time and date of the incident; (6) whether children were
2134 involved or whether the alleged act of family violence was committed
2135 in the presence of children; (7) type and extent of the alleged abuse; (8)

2136 existence of substance [abuse] use; (9) number and types of weapons
2137 involved; (10) existence of any prior court orders; (11) any other data
2138 that may be necessary for a complete analysis of all circumstances
2139 leading to the arrest.

2140 Sec. 99. Subparagraph (A) of subdivision (2) of section 46b-129a of the
2141 general statutes is repealed and the following is substituted in lieu
2142 thereof (*Effective October 1, 2026*):

2143 (2) (A) A child shall be represented by counsel knowledgeable about
2144 representing such children who shall be assigned to represent the child
2145 by the office of Chief Public Defender, or appointed by the court if there
2146 is an immediate need for the appointment of counsel during a court
2147 proceeding. Such assignment or appointment shall continue for the
2148 duration of any such proceeding under section 46b-129,
2149 notwithstanding such child's attainment of eighteen years of age. If the
2150 child's parent or guardian has been accused by a competent witness of
2151 abusing the child, or of causing the child to be neglected or uncared for,
2152 upon the assignment or appointment of counsel, such counsel shall be
2153 granted immediate access to (i) records relating to the child, including,
2154 but not limited to, Department of Social Services records and medical,
2155 mental health and substance [abuse] use treatment, law enforcement
2156 and educational records, without the necessity of securing further
2157 releases, and (ii) the child, for the purpose of consulting with the child
2158 privately. The court shall give the parties prior notice of such
2159 assignment or appointment. Counsel for the child shall act solely as
2160 attorney for the child.

2161 Sec. 100. Subsection (a) of section 51-81d of the general statutes is
2162 repealed and the following is substituted in lieu thereof (*Effective October*
2163 *1, 2026*):

2164 (a) The Superior Court, in accordance with rules established by the
2165 judges of the Superior Court, may (1) establish a Client Security Fund to
2166 (A) reimburse claims for losses caused by the dishonest conduct of
2167 attorneys admitted to the practice of law in this state and incurred in the
2168 course of an attorney-client relationship, (B) provide for crisis

2169 intervention and referral assistance to attorneys admitted to the practice
2170 of law in this state who [suffer from alcohol or other substance abuse
2171 problems or gambling problems, or who have behavioral health
2172 problems] have a mental health or substance use disorder or identified
2173 behavioral health needs, and (C) make grants-in-aid to the organization
2174 administering the program for the use of interest earned on lawyers'
2175 clients' funds accounts pursuant to section 51-81c, for the purpose of
2176 funding the delivery of legal services to the poor, and (2) assess any
2177 person admitted as an attorney by the Superior Court, in accordance
2178 with section 51-80, an annual fee to be deposited in the Client Security
2179 Fund for the purposes described in this subsection. Such crisis
2180 intervention and referral assistance (i) shall be provided with the
2181 assistance of an advisory committee, to be appointed by the Chief Court
2182 Administrator, that includes one or more behavioral health
2183 professionals, and (ii) shall not be deemed to constitute the practice of
2184 medicine or mental health care.

2185 Sec. 101. Section 51-181b of the general statutes is repealed and the
2186 following is substituted in lieu thereof (*Effective October 1, 2026*):

2187 (a) The Chief Court Administrator may establish in any court location
2188 or juvenile matters court location a docket separate from other criminal
2189 or juvenile matters for the hearing of criminal or juvenile matters in
2190 which a defendant is a drug-dependent person, as defined in section
2191 21a-240. The docket shall be available to offenders who could benefit
2192 from placement in a substance [abuse] use treatment program.

2193 (b) The Chief Court Administrator shall establish, within the
2194 appropriations designated in public act 03-1 of the June 30 special
2195 session* for said purpose, one or more drug courts for the hearing of
2196 criminal or juvenile matters in which a defendant is a drug-dependent
2197 person, as defined in section 21a-240, who could benefit from placement
2198 in a substance [abuse] use treatment program.

2199 Sec. 102. Subdivision (9) of section 53a-65 of the general statutes is
2200 repealed and the following is substituted in lieu thereof (*Effective October*
2201 *1, 2026*):

2202 (9) "Psychotherapist" means a physician, psychologist, nurse,
2203 [substance abuse] alcohol and drug counselor, social worker,
2204 clergyman, marital and family therapist, mental health service provider,
2205 hypnotist or other person, whether or not licensed or certified by the
2206 state, who performs or purports to perform psychotherapy.

2207 Sec. 103. Subsections (a) to (c), inclusive, of section 54-36i of the
2208 general statutes are repealed and the following is substituted in lieu
2209 thereof (*Effective October 1, 2026*):

2210 (a) There is established and created an account of the General Fund
2211 to be known as the "drug assets forfeiture revolving account" for the
2212 purpose of providing funds for substance [abuse] use treatment and
2213 education programs and for use in the detection, investigation,
2214 apprehension and prosecution of persons for the violation of the laws
2215 pertaining to the illegal manufacture, sale, distribution or possession of
2216 controlled substances.

2217 (b) The account shall consist of the proceeds from the sale of property
2218 and moneys received and deposited pursuant to section 54-36h.

2219 (c) Moneys in such account shall be distributed as follows: (1) Seventy
2220 per cent shall be allocated to the Department of Emergency Services and
2221 Public Protection and local police departments pursuant to subsection
2222 (d) of this section, fifteen per cent of which shall be used for purposes of
2223 drug education and eighty-five per cent of which shall be used for the
2224 detection, investigation, apprehension and prosecution of persons for
2225 the violation of laws pertaining to the illegal manufacture, sale,
2226 distribution or possession of controlled substances and for the purposes
2227 of police training on gang-related violence as required by section 7-294l,
2228 (2) twenty per cent shall be allocated to the Department of Mental
2229 Health and Addiction Services for substance [abuse] use treatment and
2230 education programs and tobacco prevention and enforcement positions
2231 engaged in compliance activities as required by the federal government
2232 as a condition of receipt of substance [abuse] use prevention and
2233 treatment block grant funds, and (3) ten per cent shall be allocated to the
2234 Division of Criminal Justice for use in the prosecution of persons for the

2235 violation of laws pertaining to the illegal manufacture, sale, distribution
2236 or possession of controlled substances.

2237 Sec. 104. Subsections (b) to (f), inclusive, of section 54-56g of the
2238 general statutes are repealed and the following is substituted in lieu
2239 thereof (*Effective October 1, 2026*):

2240 (b) The court, after consideration of the recommendation of the state's
2241 attorney, assistant state's attorney or deputy assistant state's attorney in
2242 charge of the case, may, in its discretion, grant such application. If the
2243 court grants such application, the court shall refer such person to the
2244 Court Support Services Division for assessment and confirmation of the
2245 eligibility of the applicant and to the Department of Mental Health and
2246 Addiction Services for evaluation. The Court Support Services Division,
2247 in making its assessment and confirmation, may rely on the
2248 representations made by the applicant under oath in open court with
2249 respect to convictions in other states of offenses specified in subsection
2250 (a) of this section. Upon confirmation of eligibility and receipt of the
2251 evaluation report, the defendant shall be referred to the Department of
2252 Mental Health and Addiction Services by the Court Support Services
2253 Division for placement in an appropriate alcohol intervention program
2254 for one year, or be placed in a state-licensed substance [abuse] use
2255 treatment program. The alcohol intervention program shall include a
2256 ten-session intervention program and a fifteen-session intervention
2257 program. Any person who enters the pretrial alcohol education
2258 program shall agree: (1) To the tolling of the statute of limitations with
2259 respect to such crime, (2) to a waiver of such person's right to a speedy
2260 trial, (3) to complete ten or fifteen counseling sessions in an alcohol
2261 intervention program or successfully complete a substance [abuse] use
2262 treatment program of not less than twelve sessions pursuant to this
2263 section dependent upon the evaluation report and the court order, (4) to
2264 commence participation in an alcohol intervention program or
2265 substance [abuse] use treatment program not later than ninety days after
2266 the date of entry of the court order unless granted a delayed entry into
2267 a program by the court, (5) upon completion of participation in the
2268 alcohol intervention program, to accept placement in a substance

2269 [abuse] use treatment program upon the recommendation of a provider
2270 under contract with the Department of Mental Health and Addiction
2271 Services pursuant to subsection (f) of this section or placement in a state-
2272 licensed substance [abuse] use treatment program which meets
2273 standards established by the Department of Mental Health and
2274 Addiction Services, if the Court Support Services Division deems it
2275 appropriate, and (6) if ordered by the court, to participate in at least one
2276 victim impact panel. The suspension of the motor vehicle operator's
2277 license of any such person pursuant to section 14-227b shall be effective
2278 during the period such person is participating in the pretrial alcohol
2279 education program, provided such person shall have the option of not
2280 commencing the participation in such program until the period of such
2281 suspension is completed. If the Court Support Services Division informs
2282 the court that the defendant is ineligible for such program and the court
2283 makes a determination of ineligibility or if the program provider
2284 certifies to the court that the defendant did not successfully complete
2285 the assigned program or is no longer amenable to treatment and such
2286 person does not request, or the court denies, program reinstatement
2287 under subsection (e) of this section, the court shall order the court file to
2288 be unsealed, enter a plea of not guilty for such defendant and
2289 immediately place the case on the trial list. If such defendant
2290 satisfactorily completes the assigned program, such defendant may
2291 apply for dismissal of the charges against such defendant and the court,
2292 on reviewing the record of the defendant's participation in such
2293 program submitted by the Court Support Services Division and on
2294 finding such satisfactory completion, shall dismiss the charges. If the
2295 defendant does not apply for dismissal of the charges against such
2296 defendant after satisfactorily completing the assigned program the
2297 court, upon receipt of the record of the defendant's participation in such
2298 program submitted by the Court Support Services Division, may on its
2299 own motion make a finding of such satisfactory completion and dismiss
2300 the charges. Upon motion of the defendant and a showing of good
2301 cause, the court may extend the one-year placement period for a
2302 reasonable period for the defendant to complete the assigned program.
2303 A record of participation in such program shall be retained by the Court

2304 Support Services Division for a period of ten years from the date the
2305 court grants the application for participation in such program. The
2306 Court Support Services Division shall transmit to the Department of
2307 Motor Vehicles a record of participation in such program for each
2308 person who satisfactorily completes such program. The Department of
2309 Motor Vehicles shall maintain for a period of ten years the record of a
2310 person's participation in such program as part of such person's driving
2311 record. The Court Support Services Division shall transmit to the
2312 Department of Energy and Environmental Protection the record of
2313 participation of any person who satisfactorily completes such program
2314 who has been charged with a violation of the provisions of subsection
2315 (d) of section 15-133 or section 15-140n. The Department of Energy and
2316 Environmental Protection shall maintain for a period of ten years the
2317 record of a person's participation in such program as a part of such
2318 person's boater certification record.

2319 (c) (1) At the time the court grants the application for participation in
2320 the pretrial alcohol education program, such person shall also pay to the
2321 court a nonrefundable program fee of three hundred fifty dollars if such
2322 person is ordered to participate in the ten-session intervention program
2323 and a nonrefundable program fee of five hundred dollars if such person
2324 is ordered to participate in the fifteen-session intervention program. If
2325 the court grants the application for participation in the pretrial alcohol
2326 education program and such person is ordered to participate in a
2327 substance abuse treatment program, such person shall be responsible
2328 for the costs associated with participation in such program. No person
2329 may be excluded from either program for inability to pay such fee or
2330 cost, and the court shall waive any such fee or cost for any intervention
2331 program if such person is found eligible to have such fee or cost waived
2332 under subsection (i) of this section.

2333 (2) If the court finds that a person is indigent or unable to pay for a
2334 treatment program using the method for determining indigency
2335 described in subsection (i) of this section, the costs of such program shall
2336 be paid from the pretrial account established under section 54-56k.

2337 (3) If the court denies the application, such person shall not be
2338 required to pay the program fee. If the court grants the application and
2339 such person is later determined to be ineligible for participation in such
2340 pretrial alcohol education program or fails to complete the assigned
2341 program, the program fee shall not be refunded. All program fees shall
2342 be credited to the pretrial account established under section 54-56k.

2343 (d) If a person returns to court with certification from a program
2344 provider that such person did not successfully complete the assigned
2345 program or is no longer amenable to treatment, the provider, to the
2346 extent practicable, shall include a recommendation to the court as to
2347 whether a ten-session intervention program, a fifteen-session
2348 intervention program or placement in a state-licensed substance [abuse]
2349 use treatment program would best serve such person's needs. The
2350 provider shall also indicate whether the current program referral was
2351 an initial referral or a reinstatement to the program.

2352 (e) When a person subsequently requests reinstatement into an
2353 alcohol intervention program or a substance [abuse] use treatment
2354 program and the Court Support Services Division verifies that such
2355 person is eligible for reinstatement into such program and thereafter the
2356 court favorably acts on such request, such person shall pay a
2357 nonrefundable program fee of one hundred seventy-five dollars if
2358 ordered to complete a ten-session intervention program or two hundred
2359 fifty dollars if ordered to complete a fifteen-session intervention
2360 program, as the case may be, except as provided in subsection (i) of this
2361 section. If the court grants a person's request to be reinstated into a
2362 treatment program, such person shall be responsible for the costs, if any,
2363 associated with being reinstated into the treatment program. All
2364 program fees collected in connection with a reinstatement to an
2365 intervention program shall be credited to the pretrial account
2366 established under section 54-56k. No person shall be permitted more
2367 than two program reinstatements pursuant to this subsection.

2368 (f) The Department of Mental Health and Addiction Services shall
2369 contract with service providers, develop standards and oversee

2370 appropriate alcohol programs to meet the requirements of this section.
2371 Said department shall adopt regulations, in accordance with chapter 54,
2372 to establish standards for such alcohol programs. Any person ordered
2373 to participate in a treatment program shall do so at a state-licensed
2374 treatment program which meets the standards established by said
2375 department. Any defendant whose employment or residence makes it
2376 unreasonable to attend an alcohol intervention program or a substance
2377 [abuse] use treatment program in this state may attend a program in
2378 another state which has standards substantially similar to, or higher
2379 than, those of this state, subject to the approval of the court and payment
2380 of the application, evaluation and program fees and treatment costs, as
2381 appropriate, as provided in this section.

2382 Sec. 105. Subsections (a) to (k), inclusive, of section 54-56i of the
2383 general statutes are repealed and the following is substituted in lieu
2384 thereof (*Effective October 1, 2026*):

2385 (a) There is established a pretrial drug education and community
2386 service program for persons charged with a violation of section 21a-257,
2387 21a-267, 21a-279, as amended by this act, or 21a-279a, as amended by
2388 this act. The pretrial drug education and community service program
2389 shall include a fifteen-session drug education program and a substance
2390 [abuse] use treatment program of not less than fifteen sessions, and the
2391 performance of community service.

2392 (b) Upon application by any such person for participation in such
2393 program, the court shall, but only as to the public, order the court file
2394 sealed, and such person shall pay to the court an application fee of one
2395 hundred dollars and a nonrefundable evaluation fee of one hundred
2396 fifty dollars, except as provided in subsection (l) of this section. A person
2397 shall be ineligible for participation in such pretrial drug education and
2398 community service program if such person has twice previously
2399 participated in (1) the pretrial drug education program established
2400 under the provisions of this section in effect prior to October 1, 2013, (2)
2401 the community service labor program established under section 53a-39c,
2402 (3) the pretrial drug education and community service program

2403 established under this section, or (4) any of such programs, except that
2404 the court may allow a person who has twice previously participated in
2405 such programs to participate in the pretrial drug education and
2406 community service program one additional time, for good cause shown.
2407 The evaluation and application fee imposed under this subsection shall
2408 be credited to the pretrial account established under section 54-56k.

2409 (c) The court, after consideration of the recommendation of the state's
2410 attorney, assistant state's attorney or deputy assistant state's attorney in
2411 charge of the case, may, in its discretion, grant such application. If the
2412 court grants such application, the court shall refer such person (1) to the
2413 Court Support Services Division for confirmation of the eligibility of the
2414 applicant, (2) to the Department of Mental Health and Addiction
2415 Services for evaluation and determination of an appropriate drug
2416 education or substance [abuse] use treatment program for the first or
2417 second time such application is granted, and (3) to a state-licensed
2418 substance [abuse] use treatment program for evaluation and
2419 determination of an appropriate substance [abuse] use treatment
2420 program for the third time such application is granted, except that, if
2421 such person is a veteran, the court may refer such person to the
2422 Department of Veterans Affairs or the United States Department of
2423 Veterans Affairs, as applicable, for any such evaluation and
2424 determination. For the purposes of this subsection and subsection (d) of
2425 this section, "veteran" has the same meaning as provided in section 27-
2426 103.

2427 (d) (1) (A) Upon confirmation of eligibility and receipt of the
2428 evaluation and determination required under subsection (c) of this
2429 section, such person shall be placed in the pretrial drug education and
2430 community service program and referred by the Court Support Services
2431 Division for the purpose of receiving appropriate drug education
2432 services or substance [abuse] use treatment program services, as
2433 recommended by the evaluation conducted pursuant to subsection (c)
2434 of this section and ordered by the court, to the Department of Mental
2435 Health and Addiction Services or to a state-licensed substance [abuse]
2436 use treatment program for placement in the appropriate drug education

2437 or substance [abuse] use treatment program, except that, if such person
2438 is a veteran, the division may refer such person to the Department of
2439 Veterans Affairs or the United States Department of Veterans Affairs,
2440 subject to the provisions of subdivision (2) of this subsection.

2441 (B) Persons who have been granted entry into the pretrial drug
2442 education and community service program for the first time shall
2443 participate in either a fifteen-session drug education program or a
2444 substance [abuse] use treatment program of not less than fifteen
2445 sessions, as ordered by the court on the basis of the evaluation and
2446 determination required under subsection (c) of this section. Persons
2447 who have been granted entry into the pretrial drug education and
2448 community service program for the second time shall participate in
2449 either a fifteen-session drug education program or a substance abuse
2450 treatment program of not less than fifteen sessions, as ordered by the
2451 court based on the evaluation and determination required under
2452 subsection (c) of this section. Persons who have been granted entry into
2453 the pretrial drug education and community service program for a third
2454 time shall be referred to a state-licensed substance [abuse] use program
2455 for evaluation and participation in a course of treatment as ordered by
2456 the court based on the evaluation and determination required under
2457 subsection (c) of this section.

2458 (C) Persons who have been granted entry into the pretrial drug
2459 education and community service program shall also participate in a
2460 community service program administered by the Court Support
2461 Services Division pursuant to section 53a-39c. Persons who have been
2462 granted entry into the pretrial drug education and community service
2463 program for the first time shall participate in the community service
2464 program for a period of five days. Persons who have been granted entry
2465 into the pretrial drug education and community service program for the
2466 second time shall participate in the community service program for a
2467 period of fifteen days. Persons who have been granted entry into the
2468 pretrial drug education and community service program for a third or
2469 additional time shall participate in the community service program for
2470 a period of thirty days.

2471 (D) Placement in the pretrial drug education and community service
2472 program pursuant to this section shall not exceed one year. Persons
2473 receiving substance [abuse] use treatment program services in
2474 accordance with the provisions of this section shall only receive such
2475 services at state-licensed substance [abuse] use treatment program
2476 facilities that are in compliance with all state standards governing the
2477 operation of such facilities, except that, if such person is a veteran, such
2478 person may receive services from facilities under the supervision of the
2479 Department of Veterans Affairs or the United States Department of
2480 Veterans Affairs, subject to the provisions of subdivision (2) of this
2481 subsection.

2482 (E) Any person who enters the pretrial drug education and
2483 community service program shall agree: (i) To the tolling of the statute
2484 of limitations with respect to such crime; (ii) to a waiver of such person's
2485 right to a speedy trial; (iii) to complete participation in the pretrial drug
2486 education and community service program, as ordered by the court; (iv)
2487 to commence participation in the pretrial drug education and
2488 community service program not later than ninety days after the date of
2489 entry of the court order unless granted a delayed entry into the program
2490 by the court; and (v) upon completion of participation in the pretrial
2491 drug education and community service program, to accept (I) placement
2492 in a treatment program upon the recommendation of a provider under
2493 contract with the Department of Mental Health and Addiction Services
2494 or a provider under the supervision of the Department of Veterans
2495 Affairs or the United States Department of Veterans Affairs, or (II)
2496 placement in a treatment program that has standards substantially
2497 similar to, or higher than, a program of a provider under contract with
2498 the Department of Mental Health and Addiction Services, if the Court
2499 Support Services Division deems it appropriate.

2500 (2) The Court Support Services Division may only refer a veteran to
2501 the Department of Veterans Affairs or the United States Department of
2502 Veterans Affairs for the receipt of services under the program if (A) the
2503 division determines that such services will be provided in a timely
2504 manner under standards substantially similar to, or higher than,

2505 standards for services provided by the Department of Mental Health
2506 and Addiction Services under the program, and (B) the applicable
2507 department agrees to submit timely program participation and
2508 completion reports to the division in the manner required by the
2509 division.

2510 (e) If the Court Support Services Division informs the court that such
2511 person is ineligible for the program and the court makes a determination
2512 of ineligibility or if the program provider certifies to the court that such
2513 person did not successfully complete the assigned program and such
2514 person did not request, or the court denied, reinstatement in the
2515 program under subsection (i) of this section, the court shall order the
2516 court file to be unsealed, enter a plea of not guilty for such person and
2517 immediately place the case on the trial list.

2518 (f) If such person satisfactorily completes the assigned program, such
2519 person may apply for dismissal of the charges against such person and
2520 the court, on reviewing the record of such person's participation in such
2521 program submitted by the Court Support Services Division and on
2522 finding such satisfactory completion, shall dismiss the charges. If such
2523 person does not apply for dismissal of the charges against such person
2524 after satisfactorily completing the assigned program, the court, upon
2525 receipt of the record of such person's participation in such program
2526 submitted by the Court Support Services Division, may on its own
2527 motion make a finding of such satisfactory completion and dismiss the
2528 charges. Upon motion of such person and a showing of good cause, the
2529 court may extend the placement period for a reasonable period of time
2530 to allow such person to complete the assigned program. A record of
2531 participation in such program shall be retained by the Court Support
2532 Services Division for a period of ten years from the date the court grants
2533 the application for participation in the program.

2534 (g) At the time the court grants the application for participation in the
2535 pretrial drug education and community service program, any person
2536 ordered to participate in such drug education program shall pay to the
2537 court a nonrefundable program fee of six hundred dollars. If the court

2538 orders participation in a substance [abuse] use treatment program, such
2539 person shall pay to the court a nonrefundable program fee of one
2540 hundred dollars and shall be responsible for the costs associated with
2541 such program. No person may be excluded from any such program for
2542 inability to pay such fee or cost, and the court shall waive any such fee
2543 or cost if such person is found eligible to have such fee or cost waived
2544 under subsection (l) of this section. If the court waives the costs for a
2545 substance [abuse] use treatment program, the costs of such program
2546 shall be paid from the pretrial account established under section 54-56k.
2547 If the court denies the application, such person shall not be required to
2548 pay the program fee. If the court grants the application, and such person
2549 is later determined to be ineligible for participation in such pretrial drug
2550 education and community service program or fails to complete the
2551 assigned program, the program fee shall not be refunded. All program
2552 fees shall be credited to the pretrial account established under section
2553 54-56k.

2554 (h) If a person returns to court with certification from a program
2555 provider that such person did not successfully complete the assigned
2556 program or is no longer amenable to treatment, the provider, to the
2557 extent practicable, shall include a recommendation to the court as to
2558 whether placement in a drug education program or placement in a
2559 substance [abuse] use treatment program would best serve such
2560 person's needs. The provider shall also indicate whether the current
2561 program referral was an initial referral or a reinstatement to the
2562 program.

2563 (i) When a person subsequently requests reinstatement into a drug
2564 education program or a substance [abuse] use treatment program and
2565 the Court Support Services Division verifies that such person is eligible
2566 for reinstatement into such program and thereafter the court favorably
2567 acts on such request, any person reinstated into such drug education
2568 program shall pay a nonrefundable program fee of two hundred fifty
2569 dollars, and any person reinstated into a substance [abuse] use
2570 treatment program shall be responsible for the costs, if any, associated
2571 with being reinstated into the treatment program, unless such person is

2572 found eligible to have such fee or costs waived under subsection (l) of
2573 this section. All program fees collected in connection with a
2574 reinstatement to a drug education program shall be credited to the
2575 pretrial account established under section 54-56k. No person shall be
2576 permitted more than two program reinstatements pursuant to this
2577 subsection.

2578 (j) The Department of Mental Health and Addiction Services shall
2579 develop standards and oversee appropriate drug education programs
2580 that it administers to meet the requirements of this section and may
2581 contract with service providers to provide such programs. The
2582 department shall adopt regulations, in accordance with chapter 54, to
2583 establish standards for such drug education programs.

2584 (k) Any person whose employment or residence or schooling makes
2585 it unreasonable to attend a drug education program or substance
2586 [abuse] use treatment program in this state may attend a program in
2587 another state that has standards similar to, or higher than, those of this
2588 state, subject to the approval of the court and payment of the program
2589 fee or costs as provided in this section.

2590 Sec. 106. Subsection (a) of section 54-56l of the general statutes is
2591 repealed and the following is substituted in lieu thereof (*Effective October*
2592 *1, 2026*):

2593 (a) There shall be a supervised diversionary program for persons
2594 with psychiatric disabilities, or persons who are veterans, who are
2595 accused of a crime or crimes or a motor vehicle violation or violations
2596 for which a sentence to a term of imprisonment may be imposed, which
2597 crimes or violations are not of a serious nature. For the purposes of this
2598 section, (1) "psychiatric disability" means a mental or emotional
2599 condition, other than solely substance [abuse] use disorder, that (A) has
2600 substantial adverse effects on the defendant's ability to function, and (B)
2601 requires care and treatment, and (2) "veteran" means a veteran, as
2602 defined in section 27-103, who is found, pursuant to subsection (d) of
2603 this section, to have a mental health condition that is amenable to
2604 treatment.

2605 Sec. 107. Subsection (d) of section 54-91a of the general statutes is
2606 repealed and the following is substituted in lieu thereof (*Effective October*
2607 *1, 2026*):

2608 (d) In lieu of ordering a full presentence investigation, the court may
2609 order an abridged version of such investigation, which (1) shall contain
2610 (A) identifying information about the defendant, (B) information about
2611 the pending case from the record of the court, (C) the circumstances of
2612 the offense, (D) the attitude of the complainant or victim, (E) any
2613 damages suffered by the victim, including medical expenses, loss of
2614 earnings and property loss, and (F) the criminal record of the defendant,
2615 and (2) may encompass one or more areas of the social history and
2616 present condition of the defendant, including family background,
2617 significant relationships or children, educational attainment or
2618 vocational training, employment history, financial situation, housing
2619 situation, medical status, mental health status, substance [abuse] use
2620 history, the results of any clinical evaluation conducted of the defendant
2621 or any other information required by the court that is consistent with
2622 the provisions of this section. If the court orders an abridged version of
2623 such investigation for a felony involving family violence, as defined in
2624 section 46b-38a, the abridged version of such investigation shall, in
2625 addition to the information set forth in subdivision (1) of this subsection,
2626 contain the following information concerning the defendant: (A) Family
2627 background, (B) significant relationships or children, (C) mental health
2628 status, and (D) substance [abuse] use history.

2629 Sec. 108. Subdivision (4) of subsection (f) of section 54-125a of the
2630 general statutes is repealed and the following is substituted in lieu
2631 thereof (*Effective October 1, 2026*):

2632 (4) After such hearing, the board may allow such person to go at large
2633 on parole with respect to any portion of a sentence that was based on a
2634 crime or crimes committed while such person was under eighteen years
2635 of age if the board finds that such parole release would be consistent
2636 with the factors set forth in subdivisions (1) to (4), inclusive, of
2637 subsection (c) of section 54-300 and if it appears, from all available

2638 information, including, but not limited to, any reports from the
2639 Commissioner of Correction, that (A) there is a reasonable probability
2640 that such person will live and remain at liberty without violating the
2641 law, (B) the benefits to such person and society that would result from
2642 such person's release to community supervision substantially outweigh
2643 the benefits to such person and society that would result from such
2644 person's continued incarceration, and (C) such person has demonstrated
2645 substantial rehabilitation since the date such crime or crimes were
2646 committed considering such person's character, background and
2647 history, as demonstrated by factors, including, but not limited to, such
2648 person's correctional record, the age and circumstances of such person
2649 as of the date of the commission of the crime or crimes, whether such
2650 person has demonstrated remorse and increased maturity since the date
2651 of the commission of the crime or crimes, such person's contributions to
2652 the welfare of other persons through service, such person's efforts to
2653 overcome substance [abuse, addiction] use disorder, trauma, lack of
2654 education or obstacles that such person may have faced as a child or
2655 youth in the adult correctional system, the opportunities for
2656 rehabilitation in the adult correctional system, whether the person has
2657 also applied for or received a sentence modification and the overall
2658 degree of such person's rehabilitation considering the nature and
2659 circumstances of the crime or crimes.

2660 Sec. 109. Subdivision (4) of subsection (g) of section 54-125a of the
2661 general statutes is repealed and the following is substituted in lieu
2662 thereof (*Effective October 1, 2026*):

2663 (4) After such hearing, the board may allow such person to go at large
2664 on parole with respect to any portion of a sentence that was based on a
2665 crime or crimes committed while such person was under twenty-one
2666 years of age, if the board finds that such parole release would be
2667 consistent with the factors set forth in subdivisions (1) to (4), inclusive,
2668 of subsection (c) of section 54-300 and if it appears, from all available
2669 information, including, but not limited to, any reports from the
2670 Commissioner of Correction, that (A) there is a reasonable probability
2671 that such person will live and remain at liberty without violating the

2672 law, (B) the benefits to such person and society that would result from
2673 such person's release to community supervision substantially outweigh
2674 the benefits to such person and society that would result from such
2675 person's continued incarceration, and (C) such person has demonstrated
2676 substantial rehabilitation since the date such crime or crimes were
2677 committed considering such person's character, background and
2678 history, as demonstrated by factors, including, but not limited to, such
2679 person's correctional record, the age and circumstances of such person
2680 as of the date of the commission of the crime or crimes, whether such
2681 person has demonstrated remorse and increased maturity since the date
2682 of the commission of the crime or crimes, such person's contributions to
2683 the welfare of other persons through service, such person's efforts to
2684 overcome substance [abuse, addiction] use disorder, trauma, lack of
2685 education or obstacles that such person may have faced as a person who
2686 was under twenty-one years of age in the adult correctional system, the
2687 opportunities for rehabilitation in the adult correctional system,
2688 whether the person has also applied for or received a sentence
2689 modification and the overall degree of such person's rehabilitation
2690 considering the nature and circumstances of the crime or crimes.

2691 Sec. 110. Subsection (a) of section 54-142m of the general statutes is
2692 repealed and the following is substituted in lieu thereof (*Effective October*
2693 *1, 2026*):

2694 (a) A criminal justice agency holding nonconviction information may
2695 disclose it to persons or agencies not otherwise authorized (1) for the
2696 purposes of research, evaluation or statistical analysis, or (2) if there is a
2697 specific agreement with a criminal justice agency to provide services
2698 required for the administration of criminal justice pursuant to such
2699 agreement. The Judicial Branch may disclose nonconviction information
2700 to a state agency pursuant to an agreement to provide services related
2701 to the collection of moneys due. Any such disclosure of information
2702 shall be limited to that information necessary for the collection of
2703 moneys due. Pursuant to an agreement, the Judicial Branch may
2704 disclose nonconviction information to the Department of Mental Health
2705 and Addiction Services for the administration of court-ordered

2706 evaluations and the provision of programs and services to persons with
2707 psychiatric disabilities and substance [abuse] use treatment needs.
2708 Pursuant to an agreement, the Judicial Branch may disclose
2709 nonconviction information to advocates for victims of family violence to
2710 allow such advocates to develop plans to provide for the safety of
2711 victims and victims' minor children, provided such agreement prohibits
2712 such advocates from disclosing such nonconviction information to any
2713 person, including, but not limited to, a victim of family violence.

2714 Sec. 111. Subsection (c) of section 28-5a of the general statutes is
2715 repealed and the following is substituted in lieu thereof (*Effective from*
2716 *passage*):

2717 (c) The Commissioner of Emergency Services and Public Protection
2718 shall coordinate with the [Commissioner] Commissioners of Public
2719 Health and Mental Health and Addiction Services for the deployment
2720 of grief counselors and mental health professionals to provide mental
2721 health services to the family members or other individuals with a close
2722 association with any victim of a mass shooting. Such deployments shall
2723 be made to local community outreach groups in and around the
2724 impacted geographical location and to any school or institution of
2725 higher education where any victim or perpetrator of a mass shooting
2726 event was enrolled.

2727 Sec. 112. Section 17a-470 of the general statutes is repealed and the
2728 following is substituted in lieu thereof (*Effective from passage*):

2729 Each state-operated facility for the treatment of persons with
2730 psychiatric disabilities or persons with substance use disorders, or both,
2731 shall have an advisory board appointed by the superintendent or
2732 director of the facility for terms to be decided by such superintendent or
2733 director. The superintendent or director shall appoint at least two
2734 persons with lived experience with a behavioral health disorder to the
2735 advisory board. [In any case where the present number of members of
2736 an advisory board is less than the number of members designated by
2737 the superintendent or director of the facility, the superintendent or
2738 director shall appoint additional members to such board in accordance

2739 with this section in such manner that the terms of an approximately
2740 equal number of members shall expire in each odd-numbered year. The
2741 superintendent or director shall fill any vacancy that may occur for the
2742 unexpired portion of any term. No member shall serve more than two
2743 successive terms plus the balance of any unexpired term to which the
2744 member had been appointed.] The superintendent or director of the
2745 facility shall be an ex-officio member of the advisory board. Each
2746 member of an advisory board of a state-operated facility within the
2747 Department of Mental Health and Addiction Services assigned a
2748 geographical territory shall be a resident of the assigned geographical
2749 territory. Members of said advisory boards shall receive no
2750 compensation for their services but shall be reimbursed for necessary
2751 expenses involved in the performance of their duties. [At least one-third
2752 of such members shall be from regional behavioral health action
2753 organizations, established pursuant to section 17a-484f, and at least one-
2754 third shall be members of the catchment area councils, as provided in
2755 section 17a-483, for the catchment areas served by such facility, except
2756 that members serving as of October 1, 1977, shall serve out their terms]
2757 The provisions of this section shall not apply to the Connecticut Valley
2758 Hospital Advisory Council established pursuant to section 17a-471a or
2759 the oversight board for Whiting Forensic Hospital established pursuant
2760 to section 17a-565.

2761 Sec. 113. Section 17a-471 of the general statutes is repealed and the
2762 following is substituted in lieu thereof (*Effective from passage*):

2763 Any advisory board established pursuant to section 17a-470, as
2764 amended by this act, shall: Meet with the superintendent or director of
2765 the facility periodically to advise him on the programs and policies of
2766 the facility; act as a liaison between its facility and the residents of the
2767 facility's assigned geographic territory and the state of Connecticut to
2768 inform them of the programs and policies of the facility; and issue
2769 reports, in a form and manner prescribed by such advisory board, to the
2770 Governor and Commissioner of Mental Health and Addiction Services
2771 on conditions at the facility and recommendations for changes or
2772 improvements in the facility.

2773 Sec. 114. Subsection (a) of section 17a-476 of the general statutes is
2774 repealed and the following is substituted in lieu thereof (*Effective October*
2775 *1, 2026*):

2776 (a) Any general hospital, municipality or nonprofit organization in
2777 Connecticut may apply to the Department of Mental Health and
2778 Addiction Services for funds to establish, expand or maintain
2779 psychiatric or mental health services. The application for funds shall be
2780 submitted on forms provided by the Department of Mental Health and
2781 Addiction Services, and shall be accompanied by (1) a definition of the
2782 towns and areas to be served; (2) a plan by means of which the applicant
2783 proposes to coordinate its activities with those of other local agencies
2784 presently supplying mental health services or contributing in any way
2785 to the mental health of the area; (3) a description of the services to be
2786 provided, and the methods through which these services will be
2787 provided; and (4) indication of the methods that will be employed to
2788 effect a balance in the use of state and local resources so as to foster local
2789 initiative, responsibility and participation. In accordance with
2790 subdivision (4) of section 17a-480, the regional behavioral health action
2791 organization, established pursuant to section 17a-484f, as amended by
2792 this act, serving the mental health region in which the applicant is
2793 located shall review each such application with the Department of
2794 Mental Health and Addiction Services and make recommendations to
2795 the department with respect to each such application.

2796 Sec. 115. Section 17a-482 of the general statutes is repealed and the
2797 following is substituted in lieu thereof (*Effective October 1, 2026*):

2798 As used in this section, subsection (a) of section 17a-476, as amended
2799 by this act, and sections 17a-478 to 17a-480, inclusive, as amended by
2800 this act, [and section 17a-483,] unless the context otherwise requires:
2801 "Catchment area" means any geographical area within the state
2802 established as such by the Commissioner of Mental Health and
2803 Addiction Services, the boundaries of which may be redesignated by
2804 said commissioner when deemed necessary to equalize the population
2805 of each area and in such manner as is consistent with the boundaries of

2806 the municipalities therein, provided such boundaries of any catchment
2807 area shall be entirely within the boundaries of a mental health region
2808 established under section 17a-478, as amended by this act; ["council"
2809 means the catchment area council established under section 17a-483;]
2810 "regional behavioral health action organization" means the organization
2811 established pursuant to section 17a-484f, as amended by this act; and
2812 "provider" means any person who receives income from private practice
2813 or any public or private agency which delivers mental health services.

2814 Sec. 116. Subsection (a) of section 17a-450a of the general statutes is
2815 repealed and the following is substituted in lieu thereof (*Effective October*
2816 *1, 2026*):

2817 (a) The Department of Mental Health and Addiction Services shall
2818 constitute a successor department to the Department of Mental Health.
2819 Whenever the words "Commissioner of Mental Health" are used or
2820 referred to in the following general statutes, the words "Commissioner
2821 of Mental Health and Addiction Services" shall be substituted in lieu
2822 thereof and whenever the words "Department of Mental Health" are
2823 used or referred to in the following general statutes, the words
2824 "Department of Mental Health and Addiction Services" shall be
2825 substituted in lieu thereof: 4-5, 4-38c, 4-77a, 4a-12, 4a-16, 5-142, 8-206d,
2826 10-19, 10-71, 10-76d, 17a-14, 17a-26, 17a-31, 17a-33, 17a-218, 17a-246, 17a-
2827 450, as amended by this act, 17a-451, as amended by this act, 17a-453,
2828 17a-454, 17a-455, 17a-456, as amended by this act, 17a-457, as amended
2829 by this act, 17a-458, 17a-459, 17a-464, as amended by this act, 17a-465,
2830 17a-466, 17a-467, 17a-468, 17a-470, as amended by this act, 17a-471, as
2831 amended by this act, 17a-472, 17a-473, 17a-474, 17a-476, as amended by
2832 this act, 17a-478, as amended by this act, 17a-479, 17a-480, 17a-481, 17a-
2833 482, as amended by this act, [17a-483,] 17a-498, as amended by this act,
2834 17a-499, as amended by this act, 17a-502, 17a-506, 17a-510, 17a-511, 17a-
2835 512, 17a-513, 17a-519, as amended by this act, 17a-528, as amended by
2836 this act, 17a-560, 17a-561, 17a-562, 17a-565, 17a-581, 17a-582, 17a-675, 17-
2837 836a, 17b-28, as amended by this act, 17b-59a, 17b-222, 17b-223, 17b-225,
2838 17b-359, 17b-694, as amended by this act, 19a-82, 19a-495, as amended
2839 by this act, 19a-498, 19a-507a, 19a-576, 19a-583, 20-14i, as amended by

2840 this act, 20-14j, 21a-240, 21a-301, 27-122a, 31-222, 38a-514, as amended
2841 by this act, 51-51o, 52-146h and 54-56d.

2842 Sec. 117. Section 17a-478 of the general statutes is repealed and the
2843 following is substituted in lieu thereof (*Effective October 1, 2026*):

2844 The Commissioner of Mental Health and Addiction Services shall
2845 designate mental health regions within the state. Such regions and
2846 boundaries thereof may be redesignated by said commissioner as he
2847 deems necessary. For the purposes of sections 17a-476, as amended by
2848 this act, and 17a-478 to 17a-480, inclusive, as amended by this act,
2849 "community mental health services" means comprehensive services,
2850 both medical and nonmedical, designed to (1) decrease the prevalence
2851 and incidence of psychiatric disabilities, emotional disturbance and
2852 social disfunctioning, and (2) promote mental health in individuals,
2853 groups and institutions and includes, but is not limited to, the following:
2854 Outreach and case finding, inpatient treatment, outpatient treatment,
2855 partial hospitalization, diagnosis and screening, aftercare and
2856 rehabilitation, education, consultation, emergency services, research,
2857 evaluation, training and services to the courts. The Commissioner of
2858 Mental Health and Addiction Services may enter into such contracts for
2859 services as may be required to carry out the provisions of subsection (a)
2860 of section 17a-476, as amended by this act, sections 17a-478 to 17a-480,
2861 inclusive, as amended by this act, and [sections] section 17a-482, as
2862 amended by this act. [and 17a-483.]

2863 Sec. 118. Subsections (b) to (d), inclusive, of section 17a-495 of the
2864 general statutes are repealed and the following is substituted in lieu
2865 thereof (*Effective October 1, 2026*):

2866 (b) For the purposes of this section, sections 17a-450 to [17a-483] 17a-
2867 482, inclusive, as amended by this act, 17a-496 to 17a-528, inclusive, as
2868 amended by this act, 17a-540 to 17a-550, inclusive, and 17a-560 to 17a-
2869 575, inclusive, the following terms shall have the following meanings:
2870 "Business day" means Monday to Friday, inclusive, except when a legal
2871 holiday falls on any such day; "hospital for persons with psychiatric
2872 disabilities" means any public or private hospital, retreat, institution,

2873 house or place in which any person with psychiatric disabilities is
2874 received or detained as a patient, but shall not include any correctional
2875 institution of this state; "patient" means any person detained and taken
2876 care of as a person with psychiatric disabilities; "keeper of a hospital for
2877 persons with psychiatric disabilities" means any person, body of
2878 persons or corporation which has the immediate superintendence,
2879 management and control of a hospital for persons with psychiatric
2880 disabilities and the patients therein; "support" includes all necessary
2881 food, clothing and medicine and all general expenses of maintaining
2882 state hospitals for persons with psychiatric disabilities; "indigent
2883 person" means any person who has an estate insufficient, in the
2884 judgment of the Court of Probate, to provide for his or her support and
2885 has no person or persons legally liable who are able to support him or
2886 her; "dangerous to himself or herself or others" means there is a
2887 substantial risk that physical harm will be inflicted by an individual
2888 upon his or her own person or upon another person; "gravely disabled"
2889 means that a person, as a result of mental or emotional impairment, is
2890 in danger of serious harm as a result of an inability or failure to provide
2891 for his or her own basic human needs such as essential food, clothing,
2892 shelter or safety and that hospital treatment is necessary and available
2893 and that such person is mentally incapable of determining whether or
2894 not to accept such treatment because his judgment is impaired by his
2895 psychiatric disabilities; "respondent" means a person who is alleged to
2896 have psychiatric disabilities and for whom an application for
2897 commitment to a hospital for persons with psychiatric disabilities has
2898 been filed; "voluntary patient" means any patient sixteen years of age or
2899 older who applies in writing to and is admitted to a hospital for persons
2900 with psychiatric disabilities as a person with psychiatric disabilities or
2901 any patient under sixteen years of age whose parent or legal guardian
2902 applies in writing to such hospital for admission of such patient; and
2903 "involuntary patient" means any patient hospitalized pursuant to an
2904 order of a judge of the Probate Court after an appropriate hearing or a
2905 patient hospitalized for emergency diagnosis, observation or treatment
2906 upon certification of a qualified physician.

2907 (c) For the purposes of this section and sections 17a-496 to 17a-528,

2908 inclusive, as amended by this act, "person with psychiatric disabilities"
2909 means any person who has a mental or emotional condition which has
2910 substantial adverse effects on his or her ability to function and who
2911 requires care and treatment, and specifically excludes a person who is
2912 an alcohol-dependent person or a drug-dependent person, as defined in
2913 section 17a-680.

2914 (d) For the purposes of sections 17a-453, 17a-454, 17a-456, as
2915 amended by this act, 17a-458 to 17a-464, inclusive, as amended by this
2916 act, 17a-466 to 17a-469, inclusive, 17a-471, as amended by this act, 17a-
2917 474, 17a-476 to [17a-483] 17a-482, inclusive, as amended by this act, 17a-
2918 540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-615 to
2919 17a-618, inclusive, "person with psychiatric disabilities" means any
2920 person who has a mental or emotional condition which has substantial
2921 adverse effects on his or her ability to function and who requires care
2922 and treatment, and specifically includes a person who is an alcohol-
2923 dependent person or a drug-dependent person, as defined in section
2924 17a-680.

2925 Sec. 119. Section 17a-496 of the general statutes is repealed and the
2926 following is substituted in lieu thereof (*Effective October 1, 2026*):

2927 Any keeper of a hospital for psychiatric disabilities who wilfully
2928 violates any of the provisions of this section, sections 17a-75 to 17a-83,
2929 inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as amended by this act,
2930 17a-497 to 17a-528, inclusive, as amended by this act, 17a-540 to 17a-550,
2931 inclusive, 17a-560 to 17a-575, inclusive, and 17a-615 to 17a-618,
2932 inclusive, shall be fined not more than two hundred dollars or
2933 imprisoned not more than one year or both.

2934 Sec. 120. Subsection (b) of section 17a-497 of the general statutes is
2935 repealed and the following is substituted in lieu thereof (*Effective October*
2936 *1, 2026*):

2937 (b) Upon the motion of any respondent or his or her counsel, or the
2938 probate judge having jurisdiction over such application, filed not later
2939 than three days prior to any hearing scheduled on such application, the

2940 Probate Court Administrator shall appoint a three-judge court from
2941 among the probate judges to hear such application. The judge of the
2942 Probate Court having jurisdiction over such application under the
2943 provisions of this section shall be a member, provided such judge may
2944 disqualify himself in which case all three members of such court shall
2945 be appointed by the Probate Court Administrator. Such three-judge
2946 court when convened shall have all the powers and duties set forth
2947 under sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482,
2948 inclusive, as amended by this act, 17a-495 to 17a-528, inclusive, as
2949 amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575,
2950 inclusive, and 17a-615 to 17a-618, inclusive, and shall be subject to all of
2951 the provisions of law as if it were a single-judge court. No such
2952 respondent shall be involuntarily confined without the vote of at least
2953 two of the three judges convened hereunder. The judges of such court
2954 shall designate a chief judge from among their members. All records for
2955 any case before the three-judge court shall be maintained in the Probate
2956 Court having jurisdiction over the matter as if the three-judge court had
2957 not been appointed.

2958 Sec. 121. Subsection (g) of section 17a-498 of the general statutes is
2959 repealed and the following is substituted in lieu thereof (*Effective October*
2960 *1, 2026*):

2961 (g) The hospital shall notify each patient at least annually that such
2962 patient has a right to a further hearing pursuant to this section. If the
2963 patient requests such hearing, it shall be held by the Probate Court for
2964 the district in which the hospital is located. Any such request shall be
2965 immediately filed with the appropriate court by the hospital. After such
2966 request is filed with the Probate Court, it shall proceed in the manner
2967 provided in subsections (a), (b), (c) and (f) of this section. In addition,
2968 the hospital shall furnish the Probate Court for the district in which the
2969 hospital is located on a monthly basis with a list of all patients confined
2970 in the hospital involuntarily without release for one year since the last
2971 annual review under this section of the patient's commitment or since
2972 the original commitment. The hospital shall include in such notification
2973 the type of review the patient last received. If the patient's last annual

2974 review had a hearing, the Probate Court shall, within fifteen business
2975 days thereafter, appoint an impartial physician who is a psychiatrist
2976 from the list provided by the Commissioner of Mental Health and
2977 Addiction Services as set forth in subsection (c) of this section and not
2978 connected with the hospital in which the patient is confined or related
2979 by blood or marriage to the original applicant or to the respondent,
2980 which physician shall see and examine each such patient within fifteen
2981 business days after such physician's appointment and make a report
2982 forthwith to such court of the condition of the patient on forms provided
2983 by the Probate Court Administrator. If the Probate Court concludes that
2984 the confinement of any such patient should be reviewed by such court
2985 for possible release of the patient, the court, on its own motion, shall
2986 proceed in the manner provided in subsections (a), (b), (c) and (f) of this
2987 section, except that the examining physician shall be considered one of
2988 the physicians required by subsection (c) of this section. If the patient's
2989 last annual review did not result in a hearing, and in any event at least
2990 every two years, the Probate Court shall, within fifteen business days,
2991 proceed with a hearing in the manner provided in subsections (a), (b),
2992 (c) and (f) of this section. All costs and expenses, including Probate
2993 Court entry fees provided by statute, in conjunction with the annual
2994 psychiatric review and the judicial review under this subsection, except
2995 costs for physicians appointed pursuant to this subsection, shall be
2996 established by, and paid from funds appropriated to, the Judicial
2997 Department, except that if funds have not been included in the budget
2998 of the Judicial Department for such costs and expenses, such payment
2999 shall be made from the Probate Court Administration Fund.
3000 Compensation of any physician appointed to conduct the annual
3001 psychiatric review, to examine a patient for any hearing held as a result
3002 of such annual review or for any other biennial hearing required
3003 pursuant to sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-
3004 482, inclusive, as amended by this act, 17a-495 to 17a-528, inclusive, as
3005 amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575,
3006 inclusive, and 17a-615 to 17a-618, inclusive, shall be paid by the state
3007 from funds appropriated to the Department of Mental Health and
3008 Addiction Services in accordance with rates established by the

3009 Department of Mental Health and Addiction Services.

3010 Sec. 122. Section 17a-499 of the general statutes is repealed and the
3011 following is substituted in lieu thereof (*Effective October 1, 2026*):

3012 All proceedings of the Probate Court, upon application made under
3013 the provisions of sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-
3014 483] 17a-482, inclusive, as amended by this act, 17a-495 to 17a-528,
3015 inclusive, as amended by this act, 17a-540 to 17a-550, inclusive, 17a-560
3016 to 17a-575, inclusive, and 17a-615 to 17a-618, inclusive, shall be in
3017 writing and filed in such court, and, whenever a court passes an order
3018 for the admission of any person to any state hospital for psychiatric
3019 disabilities, the court shall record the order and give a certified copy of
3020 such order and of the reports of the physicians to the person by whom
3021 such person is to be taken to the hospital, as the warrant for such taking
3022 and commitment, and shall also forthwith transmit a like copy to the
3023 Commissioner of Mental Health and Addiction Services, and, in the case
3024 of a person in the custody of the Commissioner of Correction, to the
3025 Commissioner of Correction. Whenever a court passes an order for the
3026 commitment of any person to any hospital for psychiatric disabilities, it
3027 shall, within three business days, provide the Commissioner of Mental
3028 Health and Addiction Services with access to identifying information
3029 including, but not limited to, name, address, sex, date of birth and date
3030 of commitment on all commitments ordered on and after June 1, 1998.
3031 All commitment applications, orders of commitment and commitment
3032 papers issued by any court in committing persons with psychiatric
3033 disabilities to public or private hospitals for psychiatric disabilities shall
3034 be in accordance with a form prescribed by the Probate Court
3035 Administrator, which form shall be uniform throughout the state. State
3036 hospitals and other hospitals for persons with psychiatric disabilities
3037 shall, so far as they are able, upon reasonable request of any officer of a
3038 court having the power of commitment, send one or more trained
3039 attendants or nurses to attend any hearing concerning the commitment
3040 of any person with psychiatric disabilities and any such attendant or
3041 nurse, when present, shall be designated by the court as the authority to
3042 serve commitment process issued under the provisions of sections 17a-

3043 75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as
3044 amended by this act, 17a-495 to 17a-528, inclusive, as amended by this
3045 act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-
3046 615 to 17a-618, inclusive.

3047 Sec. 123. Subsection (a) of section 17a-500 of the general statutes is
3048 repealed and the following is substituted in lieu thereof (*Effective October*
3049 *1, 2026*):

3050 (a) Each court of probate shall keep a record of the cases relating to
3051 persons with psychiatric disabilities coming before it under sections
3052 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as
3053 amended by this act, 17a-495 to 17a-528, inclusive, as amended by this
3054 act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-
3055 615 to 17a-618, inclusive, and the disposition of them. It shall also keep
3056 on file the original application and certificate of physicians required by
3057 said sections, or a microfilm duplicate of such records in accordance
3058 with regulations issued by the Probate Court Administrator. All records
3059 maintained in the courts of probate under the provisions of said sections
3060 shall be sealed and available only to the respondent or his or her counsel
3061 unless the Court of Probate, after hearing held with notice to the
3062 respondent, determines such records should be disclosed for cause
3063 shown.

3064 Sec. 124. Section 17a-501 of the general statutes is repealed and the
3065 following is substituted in lieu thereof (*Effective October 1, 2026*):

3066 Any person with psychiatric disabilities, the expense of whose
3067 support is paid by himself or by another person, may be committed to
3068 any institution for the care of persons with psychiatric disabilities
3069 designated by the person paying for such support; and any indigent
3070 person with psychiatric disabilities, not a pauper, committed under the
3071 provisions of sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483]
3072 17a-482, inclusive, as amended by this act, 17a-495 to 17a-528, inclusive,
3073 as amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575,
3074 inclusive, and 17a-615 to 17a-618, inclusive, shall be committed to any
3075 state hospital for psychiatric disabilities which is equipped to receive

3076 him, at the discretion of the Court of Probate, upon consideration of a
3077 request made by the person applying for such commitment.

3078 Sec. 125. Section 17a-504 of the general statutes is repealed and the
3079 following is substituted in lieu thereof (*Effective October 1, 2026*):

3080 Any person who wilfully and maliciously causes, or attempts to
3081 cause, or who conspires with any other person to cause, any person who
3082 does not have psychiatric disabilities to be committed to any hospital
3083 for psychiatric disabilities, and any person who wilfully certifies falsely
3084 to the psychiatric disabilities of any person in any certificate provided
3085 for in sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482,
3086 inclusive, as amended by this act, 17a-495 to 17a-528, inclusive, as
3087 amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575,
3088 inclusive, and 17a-615 to 17a-618, inclusive, and any person who, under
3089 the provisions of said sections relating to persons with psychiatric
3090 disabilities, wilfully reports falsely to any court or judge that any person
3091 has psychiatric disabilities, shall be guilty of a class D felony.

3092 Sec. 126. Section 17a-505 of the general statutes is repealed and the
3093 following is substituted in lieu thereof (*Effective October 1, 2026*):

3094 When any female with psychiatric disabilities is escorted to a state
3095 hospital for persons with psychiatric disabilities by a male guard,
3096 attendant or other employee of a correctional or reformatory institution,
3097 or by a male law enforcement officer, under the provisions of sections
3098 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as
3099 amended by this act, 17a-495 to 17a-528, inclusive, as amended by this
3100 act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-
3101 615 to 17a-618, inclusive, the person so escorting her shall be
3102 accompanied by an adult member of her family or at least one woman.

3103 Sec. 127. Section 17a-519 of the general statutes is repealed and the
3104 following is substituted in lieu thereof (*Effective October 1, 2026*):

3105 Each officer or indifferent person making legal service of any order,
3106 notice, warrant or other paper under the provisions of sections 17a-75 to

17a-83, inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as amended by this act, 17a-495 to 17a-528, inclusive, as amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-615 to 17a-618, inclusive, shall be entitled to the same compensation as is by law provided for like services in civil causes. Physicians, for examining a person alleged to have psychiatric disabilities and making a certificate as provided by said sections, shall be entitled to a reasonable compensation established by the Commissioner of Mental Health and Addiction Services. The fees of the courts of probate shall be such as are provided by law for similar services. The Superior Court, on an appeal, may tax costs at its discretion.

Sec. 128. Section 17a-525 of the general statutes is repealed and the following is substituted in lieu thereof (*Effective October 1, 2026*):

Any person aggrieved by an order, denial or decree of a Probate Court under sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as amended by this act, 17a-495 to 17a-528, inclusive, as amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-615 to 17a-618, inclusive, including any relative or friend, on behalf of any person found to have psychiatric disabilities, shall have the right of appeal in accordance with sections 45a-186 to 45a-193, inclusive. On the trial of an appeal, the Superior Court may require the state's attorney or, in the state's attorney's absence, some other practicing attorney of the court to be present for the protection of the interests of the state and of the public.

Sec. 129. Subsection (a) of section 17a-528 of the general statutes is repealed and the following is substituted in lieu thereof (*Effective October 1, 2026*):

(a) When any person is found to have psychiatric disabilities, and is committed to a state hospital for psychiatric disabilities, upon proceedings had under sections 17a-75 to 17a-83, inclusive, 17a-450 to [17a-483] 17a-482, inclusive, as amended by this act, 17a-495 to 17a-528, inclusive, as amended by this act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, and 17a-615 to 17a-618, inclusive, all fees and

3140 expenses incurred upon the probate commitment proceedings, payment
3141 of which is not otherwise provided for under said sections, shall be paid
3142 by the state within available appropriations from funds appropriated to
3143 the Department of Mental Health and Addiction Services in accordance
3144 with rates established by said department; and, if such person is found
3145 not to have psychiatric disabilities, such fees and expenses shall be paid
3146 by the applicant.

3147 Sec. 130. Subsection (d) of section 45a-656 of the general statutes is
3148 repealed and the following is substituted in lieu thereof (*Effective October*
3149 *1, 2026*):

3150 (d) The conservator of the person shall not have the power or
3151 authority to cause the respondent to be committed to any institution for
3152 the treatment of the mentally ill except under the provisions of sections
3153 17a-75 to 17a-83, inclusive, 17a-456 to [17a-483] 17a-482, inclusive, as
3154 amended by this act, 17a-495 to 17a-528, inclusive, as amended by this
3155 act, 17a-540 to 17a-550, inclusive, 17a-560 to 17a-575, inclusive, 17a-615
3156 to 17a-618, inclusive, and 17a-621 to 17a-664, inclusive, and chapter 359.

3157 Sec. 131. Subsection (e) of section 45a-677 of the general statutes is
3158 repealed and the following is substituted in lieu thereof (*Effective October*
3159 *1, 2026*):

3160 (e) A plenary guardian or limited guardian shall not have the power
3161 or authority: (1) To cause the protected person to be admitted to any
3162 institution for treatment of the mentally ill, except in accordance with
3163 the provisions of sections 17a-75 to 17a-83, inclusive, 17a-456 to [17a-
3164 483] 17a-482, inclusive, as amended by this act, 17a-495 to 17a-528,
3165 inclusive, as amended by this act, 17a-540 to 17a-550, inclusive, 17a-560
3166 to 17a-575, inclusive, 17a-615 to 17a-618, inclusive, and 17a-621 to 17a-
3167 664, inclusive, and chapter 420b; (2) to cause the protected person to be
3168 admitted to any training school or other facility provided for the care
3169 and training of persons with intellectual disability if there is a conflict
3170 concerning such admission between the guardian and the protected
3171 person or next of kin, except in accordance with the provisions of
3172 sections 17a-274 and 17a-275; (3) to consent on behalf of the protected

3173 person to a sterilization, except in accordance with the provisions of
3174 sections 45a-690 to 45a-700, inclusive; (4) to consent on behalf of the
3175 protected person to psychosurgery, except in accordance with the
3176 provisions of section 17a-543; (5) to consent on behalf of the protected
3177 person to the termination of the protected person's parental rights,
3178 except in accordance with the provisions of sections 45a-706 to 45a-709,
3179 inclusive, 45a-715 to 45a-718, inclusive, 45a-724 to 45a-737, inclusive,
3180 and 45a-743 to 45a-757, inclusive; (6) to consent on behalf of the
3181 protected person to the performance of any experimental biomedical or
3182 behavioral medical procedure or participation in any biomedical or
3183 behavioral experiment, unless it (A) is intended to preserve the life or
3184 prevent serious impairment of the physical health of the protected
3185 person, (B) is intended to assist the protected person to regain the
3186 protected person's abilities and has been approved for the protected
3187 person by the court, or (C) has been (i) approved by a recognized
3188 institutional review board, as defined by 45 CFR 46, 21 CFR 50 and 21
3189 CFR 56, as amended from time to time, which is not a part of the
3190 Department of Developmental Services, (ii) endorsed or supported by
3191 the Department of Developmental Services, and (iii) approved for the
3192 protected person by such protected person's primary care physician; (7)
3193 to admit the protected person to any residential facility operated by an
3194 organization by whom such guardian is employed, except in accordance
3195 with the provisions of section 17a-274; (8) to prohibit the marriage or
3196 divorce of the protected person; and (9) to consent on behalf of the
3197 protected person to an abortion or removal of a body organ, except in
3198 accordance with applicable statutory procedures when necessary to
3199 preserve the life or prevent serious impairment of the physical or mental
3200 health of the protected person.

3201 Sec. 132. Subsection (a) of section 17a-486 of the general statutes is
3202 repealed and the following is substituted in lieu thereof (*Effective October*
3203 *1, 2026*):

3204 (a) Prior to the arraignment of a person charged with the commission
3205 of a misdemeanor or felony, the Department of Mental Health and
3206 Addiction Services shall, to the maximum extent possible within the

3207 limits of available appropriations, with the consent of the arrested
 3208 person, cause a clinical assessment to be performed of any person who
 3209 has previously received mental health services or treatment for
 3210 substance [abuse] use from the department or who would reasonably
 3211 benefit from such services to determine whether such person should be
 3212 referred for community-based mental health services. If the person is
 3213 determined to be in need of such services and is willing to accept the
 3214 services offered, the court shall be informed of the result of the
 3215 assessment and the recommended treatment plan for consideration by
 3216 the court in the disposition of the criminal case.

3217 Sec. 133. Section 17a-483 of the general statutes is repealed. (*Effective*
 3218 *October 1, 2026*)

This act shall take effect as follows and shall amend the following sections:		
Section 1	<i>October 1, 2026</i>	4-67s
Sec. 2	<i>October 1, 2026</i>	4-67x(b)
Sec. 3	<i>October 1, 2026</i>	10-16b(a)
Sec. 4	<i>October 1, 2026</i>	10-19b
Sec. 5	<i>October 1, 2026</i>	10-220a(a)
Sec. 6	<i>October 1, 2026</i>	14-44k(h)
Sec. 7	<i>October 1, 2026</i>	17a-4(a)
Sec. 8	<i>October 1, 2026</i>	17a-22g(a)
Sec. 9	<i>October 1, 2026</i>	17a-22dd(a)
Sec. 10	<i>October 1, 2026</i>	17a-62a(b)
Sec. 11	<i>October 1, 2026</i>	17a-101j(e)
Sec. 12	<i>October 1, 2026</i>	17a-101n
Sec. 13	<i>October 1, 2026</i>	17a-450(b)
Sec. 14	<i>October 1, 2026</i>	17a-450(d)
Sec. 15	<i>October 1, 2026</i>	17a-451(a)
Sec. 16	<i>October 1, 2026</i>	17a-453c
Sec. 17	<i>October 1, 2026</i>	17a-456(a)
Sec. 18	<i>October 1, 2026</i>	17a-457(f) and (g)
Sec. 19	<i>October 1, 2026</i>	17a-464
Sec. 20	<i>October 1, 2026</i>	17a-484c
Sec. 21	<i>October 1, 2026</i>	17a-484f(b)
Sec. 22	<i>October 1, 2026</i>	17a-485d(c) to (e)

Sec. 23	October 1, 2026	17a-485i(a)
Sec. 24	October 1, 2026	17a-667(b) and (c)
Sec. 25	October 1, 2026	17a-667a(c)(1)(B)
Sec. 26	October 1, 2026	17a-667a(e)
Sec. 27	October 1, 2026	17a-670
Sec. 28	October 1, 2026	17a-673a(b)
Sec. 29	October 1, 2026	17a-683(b)
Sec. 30	October 1, 2026	17a-683(d)
Sec. 31	October 1, 2026	17a-684(a)
Sec. 32	October 1, 2026	17a-710(a)
Sec. 33	October 1, 2026	17a-750(2)
Sec. 34	October 1, 2026	17a-838(a)(8)
Sec. 35	October 1, 2026	17b-28(c)(4)
Sec. 36	October 1, 2026	17b-59d(d)(2)
Sec. 37	October 1, 2026	17b-112(c)
Sec. 38	October 1, 2026	17b-112d
Sec. 39	October 1, 2026	17b-191(c)
Sec. 40	October 1, 2026	17b-194(a)
Sec. 41	October 1, 2026	17b-195
Sec. 42	October 1, 2026	17b-241
Sec. 43	October 1, 2026	17b-241a
Sec. 44	October 1, 2026	17b-689c(a)(1)
Sec. 45	October 1, 2026	17b-694(a)
Sec. 46	October 1, 2026	18-69b
Sec. 47	October 1, 2026	18-69c(a)
Sec. 48	October 1, 2026	18-69c(g)
Sec. 49	October 1, 2026	18-87k(a)
Sec. 50	October 1, 2026	18-100f
Sec. 51	October 1, 2026	19a-6d
Sec. 52	October 1, 2026	19a-6h(e)(2)
Sec. 53	October 1, 2026	19a-7c(b)
Sec. 54	October 1, 2026	19a-7e
Sec. 55	October 1, 2026	19a-124(b)
Sec. 56	October 1, 2026	19a-490(a)
Sec. 57	October 1, 2026	19a-490h
Sec. 58	October 1, 2026	19a-490q(a)(1)
Sec. 59	October 1, 2026	19a-491(a)
Sec. 60	October 1, 2026	19a-495(d)
Sec. 61	October 1, 2026	19a-495c
Sec. 62	October 1, 2026	19a-509e
Sec. 63	October 1, 2026	19a-581(11) and (12)

Sec. 64	October 1, 2026	19a-630(10)
Sec. 65	October 1, 2026	19a-638(a)(5)
Sec. 66	October 1, 2026	19a-902
Sec. 67	October 1, 2026	20-14i
Sec. 68	October 1, 2026	20-74s(a)(4) to (6)
Sec. 69	October 1, 2026	20-74ss
Sec. 70	October 1, 2026	20-94d(b)
Sec. 71	October 1, 2026	20-162cc
Sec. 72	October 1, 2026	20-185m
Sec. 73	October 1, 2026	20-195dd(a) and (b)
Sec. 74	October 1, 2026	20-195ee
Sec. 75	October 1, 2026	20-195tt
Sec. 76	October 1, 2026	20-195kkk
Sec. 77	October 1, 2026	20-195qqq
Sec. 78	October 1, 2026	20-206bb(h)
Sec. 79	October 1, 2026	20-206nn
Sec. 80	October 1, 2026	20-660(h)
Sec. 81	October 1, 2026	21a-252(a)
Sec. 82	October 1, 2026	21a-274a(a)
Sec. 83	October 1, 2026	21a-279(a)(2)
Sec. 84	October 1, 2026	21a-279a(e)(2)
Sec. 85	October 1, 2026	21a-322(b)
Sec. 86	October 1, 2026	21a-420f(b)
Sec. 87	October 1, 2026	31-40v(b)
Sec. 88	October 1, 2026	38a-479qq(a)(6)
Sec. 89	October 1, 2026	38a-488a(a) and (b)
Sec. 90	October 1, 2026	38a-488d
Sec. 91	October 1, 2026	38a-492p
Sec. 92	October 1, 2026	38a-514(a) and (b)
Sec. 93	October 1, 2026	38a-514d
Sec. 94	October 1, 2026	38a-518p
Sec. 95	October 1, 2026	38a-999(a)(8)
Sec. 96	October 1, 2026	46a-11b(a)
Sec. 97	October 1, 2026	46a-170(f)(1)(E)
Sec. 98	October 1, 2026	46b-38d(c)
Sec. 99	October 1, 2026	46b-129a(2)(A)
Sec. 100	October 1, 2026	51-81d(a)
Sec. 101	October 1, 2026	51-181b
Sec. 102	October 1, 2026	53a-65(9)
Sec. 103	October 1, 2026	54-36i(a) to (c)
Sec. 104	October 1, 2026	54-56g(b) to (f)

Sec. 105	<i>October 1, 2026</i>	54-56i(a) to (k)
Sec. 106	<i>October 1, 2026</i>	54-56l(a)
Sec. 107	<i>October 1, 2026</i>	54-91a(d)
Sec. 108	<i>October 1, 2026</i>	54-125a(f)(4)
Sec. 109	<i>October 1, 2026</i>	54-125a(g)(4)
Sec. 110	<i>October 1, 2026</i>	54-142m(a)
Sec. 111	<i>from passage</i>	28-5a(c)
Sec. 112	<i>from passage</i>	17a-470
Sec. 113	<i>from passage</i>	17a-471
Sec. 114	<i>October 1, 2026</i>	17a-476(a)
Sec. 115	<i>October 1, 2026</i>	17a-482
Sec. 116	<i>October 1, 2026</i>	17a-450a(a)
Sec. 117	<i>October 1, 2026</i>	17a-478
Sec. 118	<i>October 1, 2026</i>	17a-495(b) to (d)
Sec. 119	<i>October 1, 2026</i>	17a-496
Sec. 120	<i>October 1, 2026</i>	17a-497(b)
Sec. 121	<i>October 1, 2026</i>	17a-498(g)
Sec. 122	<i>October 1, 2026</i>	17a-499
Sec. 123	<i>October 1, 2026</i>	17a-500(a)
Sec. 124	<i>October 1, 2026</i>	17a-501
Sec. 125	<i>October 1, 2026</i>	17a-504
Sec. 126	<i>October 1, 2026</i>	17a-505
Sec. 127	<i>October 1, 2026</i>	17a-519
Sec. 128	<i>October 1, 2026</i>	17a-525
Sec. 129	<i>October 1, 2026</i>	17a-528(a)
Sec. 130	<i>October 1, 2026</i>	45a-656(d)
Sec. 131	<i>October 1, 2026</i>	45a-677(e)
Sec. 132	<i>October 1, 2026</i>	17a-486(a)
Sec. 133	<i>October 1, 2026</i>	Repealer section

PH *Joint Favorable Subst.*

The following Fiscal Impact Statement and Bill Analysis are prepared for the benefit of the members of the General Assembly, solely for purposes of information, summarization and explanation and do not represent the intent of the General Assembly or either chamber thereof for any purpose. In general, fiscal impacts are based upon a variety of informational sources, including the analyst's professional knowledge. Whenever applicable, agency data is consulted as part of the analysis, however final products do not necessarily reflect an assessment from any specific department.

OFA Fiscal Note**State Impact:** None**Municipal Impact:** None**Explanation**

The bill makes technical, conforming and other changes that have no fiscal impact.

The Out Years**State Impact:** None**Municipal Impact:** None

OLR Bill Analysis**sHB 5517*****AN ACT CONCERNING THE DEPARTMENT OF MENTAL HEALTH AND ADDICTION SERVICES' RECOMMENDATIONS REGARDING RECOVERY-FRIENDLY LANGUAGE AND VARIOUS REVISIONS TO MENTAL HEALTH AND ADDICTION STATUTES.*****SUMMARY**

This bill changes terminology in various mental health and addiction-related statutes to language that is intended to be person-centered, and recovery-friendly. It primarily does so by replacing the terms (1) "substance abuse" with "substance use," (2) "opiate or opioid addiction" with "opioid use disorder," (3) "detoxification" with "withdrawal management," and (4) "substance abuse counselor" with "alcohol and drug counselor."

Additionally, the bill:

1. requires the Department of Emergency Services and Public Protection (DESPP) to coordinate with the Department of Mental Health and Addiction Services (DMHAS), in addition to the Department of Public Health (DPH) as under current law, when deploying behavioral health professionals after mass shootings (§ 111);
2. updates the membership requirements of certain DMHAS advisory boards to reflect current practice (§§ 112-114); and
3. repeals a statutory provision establishing catchment area councils (community councils that help plan mental health services in a geographic area) and makes related technical changes, removing statutory references to these councils (in practice, their functions are now performed by Regional Behavioral Health Action Organizations) (§§ 115-132)).

EFFECTIVE DATE: October 1, 2026, except the provisions on (1) deploying behavioral health professionals after mass shootings and (2) advisory boards for DMHAS-operated facilities take effect upon passage.

§ 111 — DEPLOYING BEHAVIORAL HEALTH PROFESSIONALS AFTER MASS SHOOTINGS

The bill requires the DESPP commissioner to coordinate with the DMHAS commissioner, in addition to the DPH commissioner as under current law, when deploying grief counselors and mental health professionals to provide mental health services to mass shooting victims' family members and other people with a close association to them.

By law, these deployments must be made to (1) local community outreach groups in and around the impacted geographic location and (2) any school or higher education institution where a mass shooting victim or perpetrator was enrolled.

§§ 112-114 — DMHAS ADVISORY BOARDS

Advisory Boards for State-Operated Facilities

Existing law requires all hospitals and facilities DMHAS operates (partially or wholly) that treat people with psychiatric disabilities or substance use disorders (or both) to have an advisory board that includes members with lived experience of behavioral health disorders (see BACKGROUND). The bill exempts from this requirement the Connecticut Valley Hospital advisory council and Whiting Forensic Hospital oversight board.

By law, facilities must have their superintendent or director make advisory board member appointments, at least two of whom must have lived experience with behavioral health disorders. The bill eliminates current law's requirement that the superintendent or director appoint additional board members when the current membership is less than the number of members they designated and to fill vacancies.

It also eliminates the requirement under current law that (1)

appointed members' terms be staggered so that an approximately equal number of member terms expire in odd-numbered years, (2) members serve no more than two successive terms in addition to any unexpired term they were appointed to fill, and (3) at least one-third of members each represent regional behavioral health action organizations and catchment area councils, which the bill repeals.

The bill also makes related minor and technical changes.

BACKGROUND

DMHAS-Operated Facilities and Advisory Boards

By law, DMHAS-operated facilities generally include the Capitol Region Mental Health Center, the Connecticut Valley Hospital (including its Addictions and General Psychiatric divisions), the Whiting Forensic Hospital, the Connecticut Mental Health Center, the Franklin S. DuBois Center, the Greater Bridgeport Community Mental Health Center, and River Valley Services (CGS § 17a-458).

A DMHAS-operated facility's advisory board must (1) periodically meet with the facility superintendent or director to advise on the facility's programs and policies, (2) act as a liaison between the facility and residents of its assigned geographic area, and (3) issue reports to the governor and DMHAS commissioner on facility conditions and make recommendations for changes or improvements (CGS § 17a-471).

COMMITTEE ACTION

Public Health Committee

Joint Favorable Substitute

Yea 21 Nay 11 (03/23/2026)